01 March 2026

Portfolio Rationales: MARCH 2026

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Strong rebound and good performance across both realised and unrealised positions

Sector outlook The biotech index (XBI) has continued to stall, and has traded flat in February 2026. This was driven by a comparative lack of new data and no major deals. Moreover, the performance of the S&P which sold off 1% in Feb has also weighed on the market more broadly. However, the resignation of CBER director Vinay Prasad, a volatile and controversial pick for the position, in March should generate performance in March. Moreover, we see slightly more catalysts for Mar – more positive data in Mar paired with the firing of Vinay Prasad should set up Mar to be strong.

Key updates

Important news. Please pay attention to our key updates on NKTR, VIR and LRMR which have major (positive) changes and have been major drivers of performance across the portfolio. We would also like to highlight QURE, EYPT, OCUL, PEPG which have had some negative developments.

Allocation. We have committed a significant portion of the portfolio towards large cap/high quality, and ph3 assets over this month. This is partially related to underperformance of larger names during the earnings season, which we felt was unrelated to fundamentals. We have allocated 39.11% to Large-Cap, 12.30% to Commercial, 19.54% to Phase 3, 9.95% to Phase 2 and 12.04% to Phase 1 assets. We reserve no cash.

Initiating positions

We have opened nine new positions and allocated all available resources. Please check out our initiating coverage on AGIO, BBIO, DNTH, FULC, INSM, LQDA, TERN, VIR, WVE. These positions significantly increase our exposure to M&A potential as well as XBI beta, which we feel will pick up as we head into Q2.

Closed positions and realised profit . We have realised over 10,000 USD of net gains over 15 positions, offsetting some major losses with even bigger victories. We have fully exited our positions in seven companies.

How to use this report?

Structure. First we walk through our new positions, then provide updates on continuing positions and close with justifications for exiting positions.

One-pagers. Our one-page sumaries are designed to give you with the essentials. The first paragraph serves as an executive summary of the most important things to note, followed by a company overview and closing with a valuation of the stock.

Updates. Any information which is new to our rationales (vs our Feb rationales) is highlighted in green

Technical Definitions. All technicals are underlined and defined in Appendix

Key Data for month ending Feb 27

Opening balance	555,270
Closing balance	570,430
Total positions	22
Average position size	4.54%
New positions	9
Closed	7

Summary

Ticker	Status	Weight (%)
ABEO	Unchanged	6.99%
ABVX	Unchanged	11.76%
AGIO	Initiated	4.80%
BBIO	Initiated	2.53%
CGEM	Unchanged	5.73%
CYTK	Unchanged	3.00%
DNTH	Initiated	3.40%
FULC	Initiated	2.22%
GRCE	Unchanged	2.44%
INSM	Initiated	1.65%
LQDA	Initiated	12.25%
NKTR	Increased	4.95%
PCVX	Increased	3.66%
PEPG	Unchanged	5.88%
RVMD	Unchanged	1.09%
RZLT	Decreased	3.66%
SLNO	Unchanged	6.54%
TERN	Initiated	10.50%
VIR	Initiated	2.03%
WVE	Initiated	6.99%
Cash		3.87%

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February 2026 performance

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	10 Mar	28 Feb	31 Jan
Balance	506.31	490.95	463.25
Gain (loss)	**3.13%**	**5.98%**	

Holdings as of reporting date (10 Mar), unrealised P/L since position inception

Ticker		Position	Shares		Cost		Current Px	P/L ($)
ABEO	$	41,280.00	8000	$	5.10	$	5.16	480.00
ABVX	$	65,766.00	600	$	116.73	$	109.61	-4272.00
AGIO	$	28,530.00	1000	$	27.07	$	28.53	1460.00
BBIO	$	13,128.00	200	$	71.36	$	65.64	-1144.00
CGEM	$	34,600.60	2260	$	10.55	$	15.31	10757.60
CYTK	$	17,165.50	275	$	63.27	$	62.42	-233.75
FULC	$	19,850.00	2500	$	11.59	$	7.94	-9125.00
GRCE	$	12,969.00	3300	$	3.50	$	3.93	1419.00
INSM	$	14,313.00	100	$	158.40	$	143.13	-1527.00
LQDA	$	8,967.50	250	$	34.75	$	35.87	280.00
NKTR	$	69,150.00	1000	$	73.09	$	69.15	-3940.00
NVO	$	11,934.00	300	$	37.61	$	39.78	651.00
PCVX	$	29,660.00	500	$	53.53	$	59.32	2895.00
PEPG	$	19,495.00	3500	$	5.62	$	5.57	-175.00
RVMD	$	34,926.50	350	$	98.77	$	99.79	357.00
RZLT	$	6,520.00	2000	$	3.35	$	3.26	-180.00
SLNO	$	21,078.10	530	$	47.07	$	39.77	-3869.00
TENX	$	2,755.00	250	$	11.48	$	11.02	-115.00
TERN	$	37,646.50	850	$	40.67	$	44.29	3077.00
UTHR	$	13,229.25	25	$	509.00	$	529.17	504.25
VIR	$	61,250.00	6250	$	9.09	$	9.80	4437.50
WVE	$	11,562.30	870	$	13.56	$	13.29	-234.90
Cash	$	300.00						
Total	**$**	**576,076.25**						

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01 March 2026

February 2026 performance

	Position	Shares	Cost	Proceeds	P/L ($)	P/L (%)
DNTH	10758	200	53.79	61.59	1560.00	14.50%
LQDA	8687.5	250	34.75	33.50	-312.50	-3.60%
QURE	11647.98	489	23.82	17.86	-2914.44	-25.02%
IKT	18700	10000	1.87	1.99	1200.00	6.42%
LRMR	23152.5	6750	3.43	3.9	3172.50	13.70%
CORT	9408	300	31.36	36.55	1557.00	16.55%
ETON	10569	650	16.26	17.27	656.50	6.21%
EYPT	23628.93	1409	16.77	13.16	-5086.49	-21.53%
OCUL	23374.32	2022	11.56	6.41	-10413.30	-44.55%
NKTR	19339.2	510	37.92	61.90	12229.80	63.24%
TECX	18848	950	19.84	21.05	1149.50	6.10%
SPRB	11032	140	78.8	81.79	418.60	3.79%
VIR	25035.46	3385	7.396	7.681	964.73	3.85%
PCVX	10706	200	53.53	60.75	1444.00	13%
PEPG	21480	4000	5.37	6.59	4880.00	23%
Total					10505.90	

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New Positions

01 March 2026

AGIOS PHARMACEUTICALS (AGIO)

Buy | $30.23 | PT $34.00|

Initiating Coverage

Blatant mispricing of commercial stage haematology franchise

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Company overview

Assets and indications Agios Pharmaceuticals is launching mitapivat in thalassemia (recent approval), wich is also approved for Pyruvate Kinase (PK) deficiency (for several years). AGIO is already looking to file a New Drug Application (NDA) for Sickle Cell Disease (SCD).

Other assets. AGIO are also advancing a next-generation PK activator (Tebapivat), although it is in a very early stage.

RISE UP phase 3: AGIO hit decisively on haemoglobin (Hb) but did not manage to hit on the other co-primary (Sickle Cell Pain Crises). Chronic hemolytic anemia is a driver of progressive organ damage, so the drug will induce benefit.

FDA alignment. AGIO is looking for regulatory alignment with the FDA, with a pre-sNDA meeting scheduled for Q1. The outcome of this meeting will determine whether the drug has a straightforward path to market based on its current data. Moreover, there is precedent with voxelotor, which was approved despite missing on VOCs as well. Like voxelotor, mitapivat met the haemoglobin response co-primary endpoint.

Valuation

PYRUKYND (mitapivat in PK) was approved in 2022 and generates <$50M in sales. We can consider this to be worth c.$100M

AQVESME (mitapivat in alpha/beta thalassemia) was approved in Dec 2025 and we can earmark $250M in peak sales potential. At a 2x multiple, AQVESME is worth a further $500M.

Little value assigned to SCD. If mitapivat is approved in SCD, sales would quickly reach the low hundreds of millions per year and potentially ramp north of $500M in annualised sales. An approved drug in SCD, voxelotor, did $300-400M with a significantly worse profile.

SCD approval is worth >$1B, risk adjusted to $400M. If we bake in a 40% PoS, (which is conservative considering the drug met statistical significance on a primary endpoint) , and use a 2x multiple we can value mitapivat in SCD comes out to $400M.

Putting it together. In sum$2B in market cap ($1B + in EV). This comes out to $34 per share, which is our price target.

Tebapivat as unmodelled upside. Due to a lack of data and an uncertain efficacy/safety profile we are not confident in assigning a value.

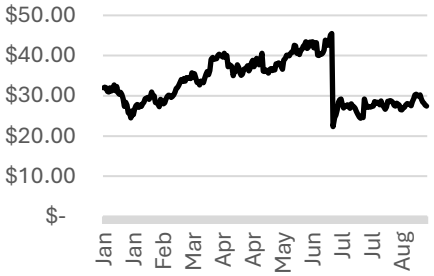
Key Data

Price	$	30
Shares (M)	$	59
MC (M)	$	1,771
Cash (M)	$	1164
Debt (M)	$	40
EV (M)	$	647

Profile

Lead Asset	Mitapivat
Phase	Commercial
Peak Sales ($B)	0.5
Price Target	$34
Catalyst	SCD regulatory alignment
Catalyst Date	Q1 2026

L12M Performance



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01 March 2026

BRIDGEBIO PHARMA (BBIO)

Buy | $66.48 | PT $85.17

Initiating Coverage

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Multi-asset high-quality biotech with moderate M&A potential and a reasonable multiple.

BBIO is a high quality biotech part of our large cap basket and our M&A basket. Surveys indicate investors see a moderate chance of a buyout and we agree – BBIO offers an attractive and reasonably valued portfolio of superb rare disease drugs. BBIO has underperformed YTD, coming off of a peak of c.$80 in mid-Jan due to PFE EU patent news overreaction. This has masked several positives including achondroplasia ph3 hit (request access) and accelerating Attruby launch (request access). Our price target of $85.17 offers significant upside of 28% without factoring in a buyout premium.

Company overview

Diversified high quality rare disease portfolio BBIO has four assets across six rare disease indications. The lead asset is ATTRUBY for ATTR cardiomyopathy (ATTR-CM), which is a best-in-class TTR stabiliser and has the best efficacy profile in the indication, despite emergent threats from ALNY's Amvuttra. In fact, all of BBIO's assets are beating their competitors on all the metrics that matter. Here's a quick breakdown

Attruby (acoramidis), approved binds to a misfolded protein (TTR) and makes it fit for its role in the myocardium. Attruby is best-in-class. The other approach to treating ATTR-CM is RNA silencing, nominally AMVUTTRA; ATTRUBY also beats AMVUTTRA on all metrics.

Infigratinib, going to be approved for achondroplasia (ACH), a type of dwarvism. Infigratinib has demonstrated superiorty on virtually over metric over the incumbents

BBP-418 for LGMD2I, a rare muscular dystrophy, has completed its trials and is awaiting approval. BBP-418 faces no challengers once on the market

Encaleret for ADH1 and CHP faces some competition in ADH1 and none in CHP, and is currently in pivotal trials which could result in upside for the stock

PFE's patent withdrawal On Feb 05, PFE withdrew their EU patent for their ATTR-CM drug tafamidis, triggering fears of earlier generic entry. ATTRUBY's significant superiority should still be able to secure reimbursement in the EU even if there is an earlier entry of a tafamidis generic. Moreover, analysts at Evercore ISI, Jefferies are sceptical that the voluntary patent withdrawal would significantly affect tafamidis protection.

Valuation

Cheap multiple and mostly de-risked data. Our SOTP yields $85.17. We assign $73.29 for Attruby + $7.57 for infigratinib, + $7.64 for encaleret + and $2.17 for BBP-418, which is partially offset by -$6.26/share in Net Debt. BBIO also owns a 17.4% stake in SpinCo BridgeBio Oncology, which is worth $0.73/share to BBIO. Altogether, this makes up a price target of $85.17. Our Peak Sales estimates are $8.2B vs Mizuho's $8.4B, Opco's $6.5B, and BBG consensus of $6.2B. We use an EV-Peak Sales multiple of 2.00x, a discount rate of 9.70% and we forecast no further dilution.

Key Data

Price	$	66
Shares (M)	$	194
MC (M)	$	12,888
Cash (M)	$	587
Debt (M)	$	1,862
EV (M)	$	14,163

Profile

Lead Asset	ATTRUBY
Phase	Commercial
Peak Sales ($B)	$8.2B
Price Target	$85.17
Catalyst	NDA submissions
Catalyst Date	1H 2026

L12M Performance

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FULCRUM THERAPEUTICS (FULC)

Buy | $8.38| PT $17

Initiating Coverage

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First Oral Disease-Modifying Therapy for Sickle Cell Disease

Company overview

Drug and market. AGIO is developing pociredir in Sickle Cell Disease (SCD), a drug that works by inducing fetal hemaglobin (HbF) in erythrocytes, in order to reduce sickling, and by proxy, VOC (vaso-occlusive crises), the main driver of morbidity and mortality in SCD. Pociredir offers a disease-modifying oral option for a very poorly managed, severe disease, and has the potential to become a blockbuster

Standard of Care So far, only gene therapies have succeeded in protecting patients against VOCs. But gene tx is prohibitively expensive, and carries the burden of liver toxicity. The go-to therapy for SCD remains hydroxyurea (HU), a very old drug with a high toxicity burden. It is modestly efficacious at reducing VOCs, but only by 10%, nowhere near the curative potential of Poceridir.

Significantly more effective than hydroxyurea. Pociredir has shown +12pp HbF improvement, bringing patients to 19.3% total HbF at 20mg dose after 12 weeks of treatment. According to natural history models, this predicts a 95% reduction in VOC incidence. While HU Hbf induction data is unclear, it has been shown to reduce the incidence of VOCs by about 40% in responder cohorts.

Incorrect Pricing of Positive Data. FULC recently released the full 12-week 20mg cohort data Likely due to some anecdotal VOC incidence data during the examined period, the market did not react favourably to the data release and share price has declined from $11.70 to $8.15.

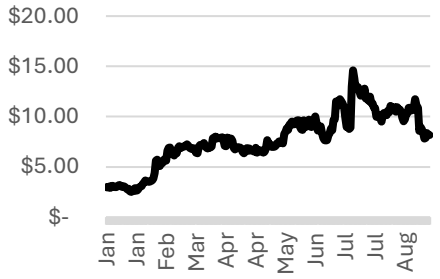
Key Data

Price	$	8
Shares (M)	$	67
MC (M)	$	558
Cash (M)	$	352
Debt (M)	$	6
EV (M)	$	212

Profile

Lead Asset	Pociredir
Phase	Ph3
Peak Sales ($B)	1.50
Price Target	17
Catalyst	N/A now
Catalyst Date	^

L12M Performance



Valuation

Peak Sales of $2.4B. There are currently 100,000 SCD patients in the US. At 40% penetration, 80% market share, 80% compliance and 75% GTN and a price point of $125,000 per year, pociredir has the potential to bring in $2.4B of peak sales.

We anticipate that at the end of Q1 2026, Fulcrum Therapeutics will have $325M in net cash. With a MC is $546M this makes roughly $200M EV.

With a 2x EV/Peak Sales multiple and any reasonable PoS, FULC remains grossly undervalued.

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INSMED INC (INSM)

Buy | $149.33

Initiating Coverage

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Large-cap commercial name available at a 30% discount from ATH.

We are initiating a position in INSM as one of our commercial/ large cap ideas. INSM holds two several large rare disease markets with no challengers. A Q1 catalyst for ARIKAYCE could see a 5x market expansion for this marketed asset whilst recent weakness can be attributed to an overreaction to a new competitor to TPIP. Overall, we do not see sufficient justification for the -32% discount to the early December all time high (ATH)

Company overview

Respiratory portfolio with blockbuster-scale potential in three assets. INSM has two commercial assets (BRINSUPRI for bronchiectasis, ARIKAYCE for MAC lung disease) and a lead pipeline asset, TPIP for pulmonary hypertension.

BRINSUPRI, approved for NCFB, is a DPP1 inhibitor and is the first treatment targeting the underlying inflammation driving NCFB, a disease with c.480k patients diagnosed and 120k eligible based on disease severity.

ARIKAYCE, approved for refractory MAC-LD, a difficult to treat chronic lung infection. Approved for refractory patients in 2018, ARIKAYCE has a TAM of about 20k patients. The upcoming ENCORE trial (Q1 catalyst) would enable an expansion into the first line (1L) setting, which would 5x the patient population to 100k+ patients.

TPIP would de-throne TYVASO in the PH market. Treprostinil Palmitil Inhalation Powder (TPIP) is a prodrug of Treprostinil (the active ingredient of TYVASO and YUTREPIA, discussed in our LQDA note). The prodrug slowly releases Treprostinil over the course of the day, enabling 1x/day (QD) dosing vs the current SoC 3x-5x/day. Our conversations with KOLs indicate that essentially "nobody would have any reason to prescribe anything other than TPIP once it hits the market with QD dosing"

Upside from soft-mist overreaction. UTHR announced they are developing Tresmi, a Treprostinil with a "soft mist inhaler" (vs the current dry powder inhaler, i.e. DPI) which could resolve up to 90% of the cough side effect that is notorious for the IP class. TPIP has an upper hand on (1) efficacy (2) compliance thanks to QD dosing (3) also milder cough than SoC due to once daily dose, and therefore we see this as an overreaction

$1B in sales guided for BRINSUPRI mgmt came out with surprise guidance exceeding all expectations indicating a fast launch for BRINSUPRI.

Valuation

Consensus peak sales of $13.3B makes for a reasonable valuation. The implied 2.2x EV-Peak Sales multiple is cheap for a high-quality multi-asset biotech and the discount is available thanks to incorrectly priced competitive threat to TPIP and a disregard for the accelerating BRINSUPRI launch

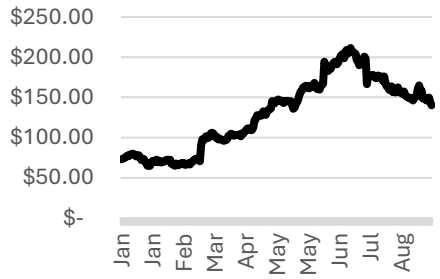
Key Data

Price	$	149
Shares (M)	$	216
MC (M)	$	32,188
Cash (M)	$	1,430
Debt (M)	$	587
EV (M)	$	31,345

Profile

Lead Asset	BRINSUPRI
Phase	Commercial
Peak Sales ($B)	$13.3B
Price Target	
Catalyst	ENCORE topline
Catalyst Date	Q1 2026

L12M Performance



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LIQUIDIA CORP. (LQDA)

Buy | $31.02

Initiating Coverage

New launch showing strong first quarters and accelerating revenue

LQDA is a new launch company trading at reasonable valuations. The market has consistently underestimated drug launches over 2025, and now we see some signs that this theme is taking shape in LQDA. Strong prescription data indicates that we will see an accelerating topline which has not been reflected in the stock price. We see litigation headwinds as mostly resolved now.

Company overview

What does LQDA do? Liquidia is launching their inhaled treprostinil, YUTREPIA for Pulmonary Arterial Hypertension (PAH) – high blood pressure in the lungs. Treprostinil binds the prostacyclin receptor (IP) which stimulates adenylyl cyclase, increasing intracellular cAMP and triggering the PKA-mediated phosphorylation that results in vasodilation and therefore reduces pulmonary hypertension and strain on the right heart as well as improved blood flow (and oxygenation).

The drug class was established by incumbent Tyvaso, produced by United Therapeutics (UTHR). Inhaled treprostanil has been used for 15 years, with Tyvaso being approved in 2009. Tyvaso is doing $1.8B in annual sales.

YUTREPIA launch Yutrepia is now in its third quarter of launch and is doing well. Estimates put the launch trajectory in line with the early progress of TYVASO DPI.

Taking significant market share two quarters into launch, YUTREPIA accounts for 17% of the market.
Differentiation between the two is in the cough. Yutrepia's dry powder inhaler (DPI) reportedly results in less coughing, which is the primary side effect. There is insufficient data at the moment to confidently claim this.

Litigation was initiated by UTHR in 2020 and is still currently ongoing. LQDA has won on most challenges, and is unlikely to result in existential threat. The current worst case for LQDA is UTHR securing damages.

Valuation

BBG consensus puts peak sales at $1.25B. The current c.$3B valuation puts the multiple at 2.4x, which is reasonable. Given the accelerating topline and strong prescription data coming out, as well as anecdotal patient stories re: cough differentiation, we feel that this is a reasonable name to hold. The key risk is the competitive landscape tightening up, especially with ralinepag (oral prostacyclin), and TPIP (INSM's PAH candidate)

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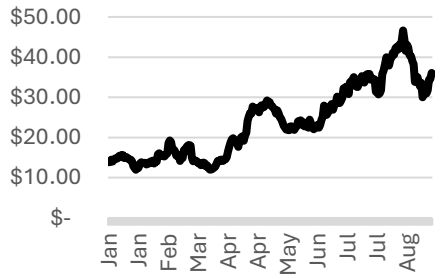
Key Data

Price	$	31
Shares (M)	$	88
MC (M)	$	2,733
Cash (M)	$	191
Debt (M)	$	7
EV (M)	$	2,550

Profile

Lead Asset	YUTREPIA
Phase	Commecial
Peak Sales ($B)	$1.25B
Price Target	
Catalyst	Q1 earnings
Catalyst Date	Q2

L12M Performance



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REVOLUTION MEDICINES (RVMD)

Buy | $102.02

Initiating Coverage

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Revolution medicines will become an oncology giant with a treatment for the deadliest cancers

RVMD is the highest quality company we know and the best acquisition target, trading at reasonable multiples because the market is slow to digest/reflect a fresh $12-24B franchise. Several acquirers have been rumoured to be bidding for RVMD and so far been unable to settle on a price. With a cheap multiple, de-risked data and good deal prospects, we are happy to include RVMD in the M&A basket.

Company overview

First treatment for PDAC. RVMD's multi-RAS inhibitor daraxonrasib is the first drug to show a very meaningful improvement in progression and survival in Pancreatic Duct AdenoCarcinoma (PDAC), one the of most deadly cancers.

What is RAS? When a growth factor hits a receptor on the cell, RAS activates the MAPK pathway that results in the cell dividing. RVMD's drugs bind to the RAS receptors in their active, bound form to stop this signalling. About 90% of PDAC patients carry a mutation in the KRAS gene that is responsible for producing the protein that regulates this signalling pathway.

Daraxonrasib is a multi-selective inhibitor which blocks the effects of many mutations. The broad coverage means it works in a broad variety of PDAC patients as well as other cancers like NSCLC and CRC. PDAC is the most advanced program.

RASOLUTE-302 : daraxonrasib in 2L metastatic PDAC. The most important trial, reading out in H1 2026. We believe the trial is likely to succeedand would require (1) daraxonrasib underperformance (2) control overperformance and (3) a particularly high censoring rate to fail. Two other trials, RASOLUTE-303 & 304 are currently enrolling to study daraxonrasib in a 1L setting and in adjuvant resected settings (respectively), but will be reading out in a long time.

Zoldonrasib is a selective inhibitor targeting KRAS G12C. A more specific therapy will have better tolerability and can be dosed more aggressively for a greater effect. There won't be any significant updates to this program this year.

Prime buyout target the RAS(ON) franchise that RVMD has developed is de-risked and massive, covers multiple indications and has strong IP. Therefore, it is a prime target for companies like MRK thinking about replacing a franchise like Keytruda.

MRK, ABBV talks. Reporting from WSJ and FT indicated that RVMD was close to a deal but was unable to close at the $28-32B valuation they were seeking. This shows MGMT is able, willing and likely to sell, but will not sell for a poor premium.

Valuation

Consensus peak sales of $12.7B makes the $17.6B EV look small. Some analysts place risk-adj. estimates even higher at $24B, $15B (Mizuho, Oppenheimer)

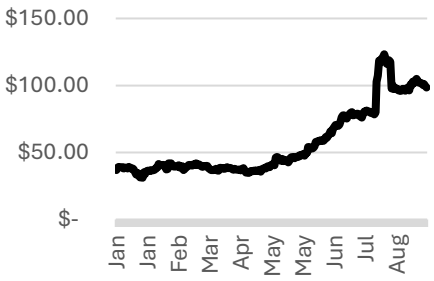
Key Data

Price	$	102
Shares (M)	$	198
MC (M)	$	20,218
Cash (M)	$	2,026
Debt (M)	$	159
EV (M)	$	18,351

Profile

Lead Asset	Daraxonrasib
Phase	3
Peak Sales ($B)	$12B-$24B
Price Target	
Catalyst	RASolute-302 topline
Catalyst Date	Q2 2026

L12M Performance



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01 March 2026

TERNS PHARMA (TERN)

Buy | $42.12

Initiating Coverage

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A better Asciminib? And no pancreatic toxicity...

TERN has a promising asset for CML, which has the potential to replace the current standard of care, scemblix which does $3-4B in sales. With a large patient data set, we view the data as relatively de-risked. Although there is not a significant discount to fair value, we add TERN to our M&A basket as we believe the odds of an acquisition are better than other investors imply.

Company overview

Drug overview. TERN has a single clinical-stage asset, TERN-701 – a next generation tyrosine kinase inhibitor being developed for Chronic Myeloid Leukemia.

CML introduction Patients with CML on standard of care (SCEMBLIX /asciminib, NVS) have a normal life expectancy. With survival no longer a problem, concerns shift to optimising for safety and tolerability. The two main efforts that emerged in 2025 were TERN-701 and ELVN-001

TERN-701 ideally positioned to become 1L.. Ph2 data has shown better response rates and broader coverage of mutations that asciminib and therefore has the potential to replace TERN-701 to replace asciminib.

ELVN-001 likely to become a second or third line treatment. ELVN-001 shows absolute response rates inferior to both asciminib and TERN-701. However, with a unique MoA, ELVN-001 will have a place in the market nonetheless for non-responders to first-line treatment.

Valuation

Sales potential Novartis (NVS) is guiding for $3-4B in sales for 2026. Moreover, the CML market is growing steadily at a c.6% CAGR. Let's assume $3B sales potential for TERN-701

Path to market. Firstly, TERN-701 must succeed in a regulatory trial, and we model a PoS of 80% given the large patient size of their Ph2 dataset. After securing a regulatory dataset, they must secure approval, which should happen with 90% PoS conditional on a good regulatory outcome. We project around 2 years until approval, and we discount at WACC of 13% over this period. Putting this together, TERN-701 is worth $3.38B. Adding in c.$1B in cash & investments, fair equity value is around c.4.38B.

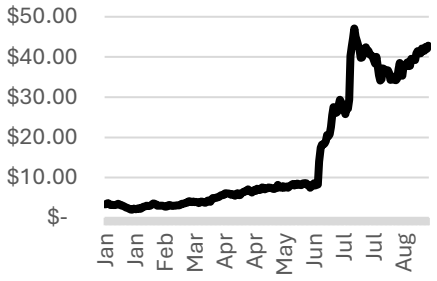
Key Data

Price	$	42
Shares (M)	$	106
MC (M)	$	4,473
Cash (M)	$	1000
Debt (M)	$	1
EV (M)	$	3474

Profile

Lead Asset	
Phase	
Peak Sales ($B)	
Price Target	
Catalyst	
Catalyst Date	

L12M Performance



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01 March 2026

WAVE LIFE SCIENCES (WVE)

Buy | $13.93

Initiating Coverage

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An old friend is back for a Q1 catalyst with 240mg 6mo data, and 400mg 3mo data.

WVE has an interesting asset for obesity that could offer significant benefits vs GLP1s. A longer duration readout could resolve some key investor questions.

Company overview

siRNA for obesity. WVE has two assets , WVE-007 and WVE-006. The former is the lead asset, which is being studied as a weight loss drug, and was responsible for the company's almost 3x transformation in December . The latter is for a rare disease called alpha-1-antitrypsin deficiency (AATD), which has some competitiveness issues.

WVE-007 is a RNA silencer which stops the function of INHBE, a gene encoding inhibin subunit βE. A naturally occurring loss of function mutation in this gene was associated with less obesity. WVE recreated this loss of function using a siRNA drug, resulting in "GLP1-like fat loss", with some caveats. WVE-007 only reduced body mass by -0.9% at 3mo, but reduced visceral (around organs) fat by -9.2%, total fat by -4.0% and preserved/increased lean mass by +0.9%.

Benefits vs GLP1 WVE-007 would (1) avoid gastrointestinal side effects (i.e don't make people vomit, feel nauseous) that plague GLP1s (2) have biannual dosing (down from QW injections) (3) preserve muscle mass (4) could work in combination in GLP1 or in a maintenance setting (5) drives impressive visceral fat reductions that GLP1s don't.

ARO-INHBE data ARWR has a very similar drug, another RNAi tx which inhibits INHBE. On Jan-07, ARWR reported their results, which were very similar to WVE-007 and further validated the modality of WVE-007, and **liver fat reduction of 38.6%**, which is higher than some MASH drugs. WVE-007 likely has the same effect, but this was not seen in the WVE study because they used DEXA instead of MRI to measure fat loss.

WVE-006 is not good enough, no value assigned. WVE competes with two companies in AATD, BEAM and ARWR. Unfortunately, despite having a decent effect which will probably result in resolution of disease progression and symptoms, both comps are better. This leaves very little space for WVE-006.

Valuation

Upside based on comps. GPCR and VKTX, both developing oral GLP1s, have EVs of $2.65B and $3.1B respectively. If the INHBE data continues to mature to plan, the significant benefits outlined above should place WVE at a premium to both, representing at least 44% upside. This upside will be realised in parts as we (1) see durability of treatment effect and (2) see more patients dosed with drug (currently the number of patients is quite low, n=24) (3) more **total** weight loss to appease regulators.

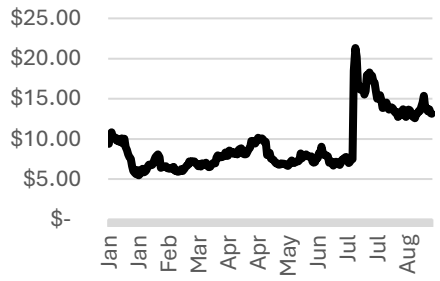
Key Data

Price	$	14
Shares (M)	$	188
MC (M)	$	2,622
Cash (M)	$	602
Debt (M)	$	18
EV (M)	$	2,038

Profile

Lead Asset	WVE-007
Phase	1
Peak Sales ($B)	
Price Target	
Catalyst	WVE-007 longer term data
Catalyst Date	Q1 2026

L12M Performance



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Continuing Positions

01 March 2026

ABEONA THERAPEUTICS (ABEO)

Buy | $5.12 | PT $8.53

Update

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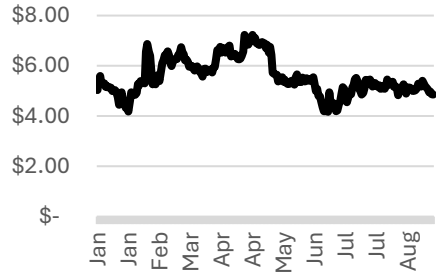
Key Data

Price	5.06
Shares (M)	52.40
MC (M)	274.2
Cash (M)	207.1
Debt (M)	24.1
EV (M)	91.2

Lead Asset	ZEVASKYN
Phase	Commercial
10Y Cumulative Revenue ($B)	$1.92B
Price Target	$8.53
Catalyst	Q4 earnings
Catalyst Date	Q1 2026

L12M Performance



Ready for a phenomenal 2026

ABEO has suffered several delays in the first quarters of their launch, which has reflected in the share price weakness. Fortunately, we see that these transient delays have now been resolved, insurance coverage has been secured (validating the value proposition), and patient demand is strong (based on our interactions). With all the pieces for a strong launch now in place, we feel that 2026 will be a very strong year – especially considering that all the delayed revenue of Q3, Q4 of 2025 should have been shifted to 2026 instead. Overall, we continue to see ABEO as a very cheap (<$100M EV) commercial play with strong differentiation, severe indication and simple commercial landscape.

Investment Thesis

Quick refresher on Abeona and the ZEVASKYN launch. Recall, ABEO is a commercial stage firm with an approved autologous gene therapy, ZEVASKYN, indicated for the treatment of Recessive Dystrophic Epidermolysis Bullosa (RDEB). Children with this debilitating genetic condition are born without the COL7A1 gene and are unable to produce collagen-7 (COL7), resulting in brittle skin prone to damage and with poor healing. Abeona contends with a single incumbent competitor, Krystal Biotech's (KRYS) VYJUVEK cream. We see strong value proposition and differentiation for ZEVASKYN as VYJUVEK is unable to close large wounds (>20 cm^2) , which result in the most patient discomfort, infections and other complications. Abeona's therapy was approved by the FDA in Q2 2025.

However, they were unable to dose their first patient until December due to (1) a manufacturing issue due to a false positive rapid sterility assay and (2) patient-level delays. The manufacturing issue meant that biopsies were delayed until November and the first patient dose was delayed from August to December. We also expect there were some individual patient delays with the first patient; first dose occurred at Lucile Packard Children's Hospital, which conflicts with guidance that this was occurring at Lurie Children's Hospital (first COE). This could indicate that there was a unique delay with the original patient at Lurie Children's. We expect these initial delays to be transient – as they clear a strong launch should emerge.

Likely to beat consensus for Q1. We have spoken with a relative of a patient with RDEB who has confirmed that she "knows of people who have dates set for March", with her child beginning the process on Jan 27. This implies at least 3 patients for Q1 dose. With just 6 patients necessary to beat Q1 consensus revenue, we feel that a likely consensus beat is coming when Q1 earnings are released in Q2. With respect to this, we have increased the ABEO allocation.

Valuation

Price target of $8.53 based on weighted average of rNPV/DCF and cumulative sales multiple . Our price target (PT) is based on a blended average (50/50) of our rNPV cash flow model PT of $8.07 and and sales-multiple price target of $8.99. For this one-time treatment configuration, we have used the lifetime cumulative sales opportunity ($1.92B) and a multiple of 0.15x, which is significantly below the historical peer group of 0.30x, reflecting the ultra orphan disease risk.

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01 March 2026

ABIVAX SA (ABVX)

Buy | $121.28 | PT $155

Update

No deal despite M&A rumours. Still a quality pick for 2026

A biotech darling in 2025, Abivax continues to mature. ABVX is the most commonly held stock amongst US biotech specialist funds. Much speculation about potential M&A has dominated early 2026 commentary on ABVX. In our view, M&A or not, Abivax still trades at a reasonable valuation and the likely prospect of M&A over the coming year introduces significant upside. Moreover, as the near the maintenance trial data in Q2 2026, we anticipate renewed interest in the name.

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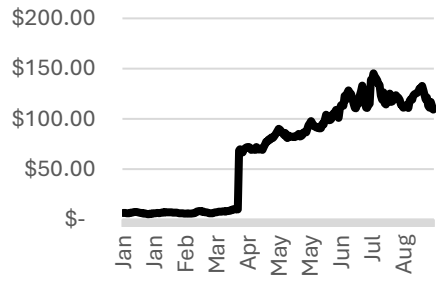
Key Data (USD)

Priche	$	121.28
Shares (M)	$	79
MC (M)	$	9,562
Cash (M)	$	152
Debt (M)	$	109
EV (M)	$	9,519

Lead Asset	obefazimod
Phase	3
Peak Sales (2032)	~$4B
Price Target	$155
Catalyst	Ph3 maintenance trial
Catalyst Date	Late Q2 2026

L12M Performance



Company overview

Product overview, market summary ABVX is a French biotech developing obefazimod (lead asset) for inflammatory bowel disease (IBD – Ulcerative Collitis & Crohn's Disease). Following strong data in Q2 2025, ABVX has rocketed to a likely best-in-class status in UC off the back of obefazimod.

Obefazimod is a once-daily oral small molecule for ulcerative colitis (UC), purportedly working by upregulating miR-124 expression to reduce inflammatory cytokines (IL-23, IL-6, IL-17), though the mechanism remains poorly understood.

Obefazimod would be the only oral UC therapy without a black box warning or severe infection risk (unlike JAK inhibitors and S1P1 modulators), and the only drug with its unique miR-124 mechanism – important for capturing non-responders to existing therapies with duplicative mechanisms. This positions it favourably against the standard of care, which is crowded with IL-23 and TL1A drugs.

Having read out Phase 3 induction results with resoundingly positive results, the thesis is significantly de-risked – it is very likely that obefazimod will make it to market. And it will become a multi-blockbuster disrupting the standard of care.

The UC space has seen several large deals in the past: tulisokibart (Prometheus acquired by Merck, 2023) sold for $10.8B, ozanimod (Receptos bought by Celgene, 2015) for $7.2B amongst others provide solid comps for an ABVX buyout. With obefazimod having a better profile vs both (tulisokibart is a subcutaneous drug, ozanimod has a black box warning), a BO is likely to happen at a significantly higher valuation, particularly when pharma is scrambling to replace multiple UC blockbusters with looming loss of exclusivity (Humira, Stelara, Entyvio, Xeljanz)

Valuation

Reasonably valued with significant upside. Consensus places obefazimod peak sales at c.$4B. This means that the current valuation represents approximately a 3x EV/Peak Sales ratio. With the average 2025 M&A happening at 6.5x projected 2030 revenue, and a strong likelihood of a BO for ABVX in 2026, ABVX remains reasonably valued at current levels with significant M&A upside.

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01 March 2026

CULLINAN THERAPEUTICS (CGEM)

Buy | $15.50 | PT $17

Update

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Becoming increasingly expensive due to a developing oncology program.

CGEM is developing a novel approach in severe refractive autoimmune disease which could disrupt the current treatment paradigm. We're inching closer towards a first in human readout of CLN-978 in SLE and RA guided towards the middle of the year. YTD, CGEM is up roughly 60% from promising data in their oncology franchise but, as before, very little value is assigned to the autoimmune franchise. We believe that CGEM is in for a strong first-in-human readout later this year that will trigger a significant re-rating. The oncology data is nice to have as well.

A novel paradigm

T-cell engagers in autoimmune & inflammatory disease
CLN-978 is a bispecificCD3*CD19 T-Cell engager (TCE) set to read out clinical trial results in the first half of 2026. CD19 is a pivot from the previously popular target of CD20, in contrast to CD20,CD19 is expressed more broadly across B-cell lineages, importantly, in the plasma cells. Cullinan's drug has shown very promising depletion data in preclinical murine models and we believe that it is likely to be a next generation drug for the treatment of systemic lupus erythematosus (SLE) and potentially rheumatoid arthritis (RA).

The modality is supported by a case series out of Germany (Muller et al. 2024) on the use of CD19 CAR-T therapy. All nine severe lupus patients achieved remission after a single CD19 CAR T infusion, setting off a race to investigate this new treatment paradigm and amassing 850+ citations in under a year. Very inspirational stuff.

CAR-T therapy involves genetically engineering a patient's own T cells to attack a specific target (historically, tumours in oncology settings). Commercially, CAR-T has met severe limitations with high manufacturing costs and high-volume manufacturing. TCEs are bispecific antibodies which bind to a T-Cell and bind to a target molecule (in this case, B cells). CD3*CD19 TCEs bind CD3 on the surface of T cells and CD19 on the surface of B cells, redirecting endogenous T cells to kill B cells. Once the patient's existing B cells (which are autoreactive, causing the autoimmune dysfunction) are wiped out, B cells regenerate de novo from stem cells without the same autoreactive properties. Since TCEs are just antibodies, they are significantly easier to implement at scale (off the shelf) whilst achieving comparable depletion.

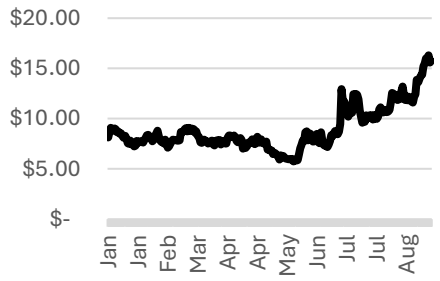
Zipalertinib and CLN-049 zipalternib is a drug that will have to complete with several other therapies in the ex20ins NSCLC class. CLN-049 is a bispecific TCE for AML that is heading into a significantly less competitive space and has produced some initial promising data.

Key Data

Price	$	15.50
Shares (M)	$	59
MC (M)	$	916
Cash (M)	$	399
Debt (M)	$	2
EV (M)	$	519

Lead Asset	CLN-978
Phase	1
Peak Sales	
Price Target	
Catalyst	Initial data in SLE and RA
Catalyst Date	2Q 2026

L12M Performance



Valuation

Unclear without efficacy profile, but real shots on goal in significant indications Without a clear efficacy profile so far, it is difficult to value CLN-978. However, it is clear that this is a potential paradigm shifting drug in a indication with billions up for grabs (Benlysta, the standard of care, sales are roughly $1.8B) which currently has very little attention and value attributed to it.

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01 March 2026

CYTOKINETICS, INC (CYTK)

Buy | $62.22| PT $77.19

Initiating Coverage

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Looking ahead to ACACIA nHCM readout

CYTK recently achieved FDA approval of its lead asset, aficamten (brand: MYQORZO) in oHCM. Analysts are looking for a strong launch and a potential hit in nHCM, which would roughly double the market. Although nHCM has proved to be a challenge for the previous generation of cardiac myosin inhibitor (mavacamten), we believe that aficamten stands a strong change to hit in this indication given lower treatment interruption rates. In the case of nHCM success, aficamten will become the first drug in this indication.

Investment Thesis

Aficamten in HCM. Hypertrophic Cardiomyopathy (HCM) is a disease in which the heart muscle becomes thickened (hypertrophied). The hypertrophy results in diastolic dysfunction (the enlarged heart is less flexible and unable to fill properly), which causes a range of cardiac symptoms like dyspnea, angina, palpitations (atrial fibrillation), syncope and long-term risk of heart failure.

Obstruction. The hypertrophy can lead to a narrowing of the left ventricle outflow tract (LVOT, the tract heading into the aorta), creating a large pressure gradient (LVOT gradient) between the aorta and left ventricle, which characterises obstructive HCM (oHCM). oHCM makes up 65-70% of cases of HCM whilst nHCM represents the rest.

Aficamten and cardiac myosin inhibitors work by reducing the contractility of the heart muscle, resolving the hypercontractility caused by the hypertrophy and therefore reducing the LVOT gradient. Aficamten works by the same mechanism as mavacamten (standard of care) but has a wider "therapeutic window" – the time during which the concentration of the drug is high enough to achieve the desired effect. This means less treatment interruption (a known problem for mavacamten) and better efficacy over time.

nHCM opportunity The main short term catalyst is going to be the ACACIA-HCM readout in nHCM. Not too long ago, mavacamten failed in this indication – but not by much. Although nHCM is a much more difficult indication (endpoint is not LVOT gradient, it is exercise capacity (pVO2)) and the impact of reducing contractility is less clear, the better pharmacokinetic profile of aficamten can help to push it over the edge and get the win that mavacamten narrowly missed.

Valuation

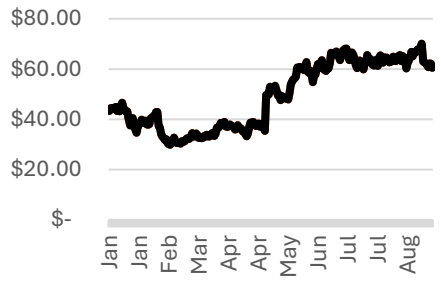
ACACIA upside seems underestimated by mkt. We model risk adjusted peak sales of $4.84B for aficamten in oHCM (approved, 100% PoS) and nHCM (Ph3, 65% PoS). This extends to a price target of $78, meaning the stock is significantly undervalued at the moment. In the case of nHCM success, we expect upside of ~$30/share (~50%).

Key Data

Price	$	62.22
Shares (M)	$	123
MC (M)	$	7,663
Cash (M)	$	882
Debt (M)	$	1,284
EV (M)	$	8,065

Lead Asset	aficamten
Phase	3/Commercial
Risk adjusted Peak Sales	$4.84B
Price Target	$77.19
Catalyst	ACACIA-HCM topline
Catalyst Date	Late Q1 – April 2026

L12M Performance



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01 March 2026

NEKTAR THERAPEUTICS (NKTR)

BUY | $68.98| PT $100

Update

Outstanding Maintenance Data Surpasses all Expectations

In our last coverage of Nektar Therapeutics, we said "We are looking towards maintenance data in AD to reverse this loss of momentum" –
This AD maintenance data publication transpired very favourably, as we anticipated.

A potential first line competitor to Dupixent?

Recap of drug and market NKTR's rezpegaldesleukin produced some proof of concept data in 2025 which made it a very strong option for the go-to second line treatment for patients who have failed the first line, Dupixent (or other IL4/13 therapies like lebrikizumab). The MoA is an IL-2 receptor agonist which selectively expands regulatory T cell (Treg) cell and CD8+ T cell activity without triggering Th2 to increase immune response. TL;DR (1) the mechanism is different, which positions it well in the 2L setting (2) in that setting, it is the most competitive data profile at the moment (3) NKTR has enough data to call it "de-risked".

Alopecia Areata update scheduled for early Q2 2026. As we mentioned, we believe that rezpeg has activity in AA and look forward to what we believe will be another positive data event for the company in Q2 of this year. Success in AA adds meaningfully to NKTR's offering but we just consider this a call option as it is **currently vastly underpriced for AtD alone.**

The stellar maintenance data – Nektar presented maintenance data for rezpeg in AtD in February. This data was so good that is raised the question of first-line competition against Dupixent in AtD because of the durability advantage.
In any case, we believe rezpeg to be ideally positioned as a pre-JAKi and at least post-Dupixent 2L option with no equal. Some other datasets from CRVS and APGE have been produced and rewarded by the market but we believe that NKTR's data package is by far the most robust.

OX40 drug class continues to disintegrate – Amlitelimab, the most serious OX40 contender by Sanofi is now associated with Kaposi's Sarcoma.

Other drugs have produced better absolute efficacy data in the clinic, but they are either 1- same MoA as Lebri/Dupi, 2 – have a tox overhang (i.e. CRVS), 3 – have an very small sample size (CRVS, APGE).

Quick note on the lawsuit All quiet on this front. NKTR seems to have the upper hand, but the process is slow and unpredictable.

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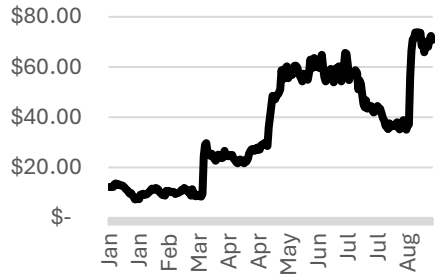
Key Data

Price	$	68.98
Shares (M)	$	27
MC (M)	$	1,882
Cash (M)	$	255
Debt (M)	$	103
EV (M)	$	1,730

Lead Asset	rezpegaldesleukin
Phase	2
Peak Sales	$1B
Price Target	
Catalyst	AA further data
Catalyst Date	

L12M Performance



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01 March 2026

VAXCYTE INC (PCVX)

Buy | $61.74| PT $74.41

Update

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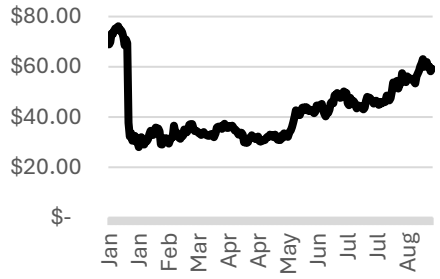
Key Data

Price	$	61.74
Shares (M)	$	144
MC (M)	$	8,886
Cash (M)	$	1,561
Debt (M)	$	117
EV (M)	$	7,442

Lead Asset	VAX-31
Phase	Ph3
US Peak Sales	$5.00B
Price Target	$74.41
Catalyst	VAX31 Ph3 Adults
Catalyst Date	Q4 2026

L12M Performance



Large and clear opportunity, but a lot of waiting

PCVX is one of the largest valuation discrepancies available in the market at the moment , both in terms of impact ($6B+ market) and value gap (check valuation). PCVX is available at a discount due to (1) macro concerns from RFK Jr commentary on vaccines and (2) an insignificant miss on some serotypes in the VAX-24 Infs data in Mar 2025. Regarding RFK Jr, the administration's (misguided) vaccine commentary does not target PCV vaccines at all, and the market reaction is overblown. Regarding the VAX-24 Infs data, it is still strong (approvable) data and a higher dose regimen is being pursued to resolve those problems. The third factor keeping the stock down is the long wait to a catalyst, which will be in Q4. This creates the opportunity for a steep run up into Q4 and 2027.

Investment Thesis

Market opportunity and drug intro. PCVX is developing a next-gen invasive pneumococcal disease vaccine / IPD (big market; ~400M+ doses annually). Current ACIP recommendations are (1) 4x doses of PCV20 for infants, (2) a single dose for adults age 19-49 with underlying medical conditions (UMC), and (3) a single dose for adults > 50 y.o. (recently updated from >65 y.o.).

The key differentiator is serotype coverage – the number of different variations (strains) of the virus covered by the vaccine. Existing incumbents, Pfizer's PREVNAR20 and 21 (i.e. PCV20 / PCV21) garner ~$6.5B of sales and hold 70-80% of the market share. Historically, the therapy with the greatest serotype coverage wins out.

The problem is that PFE and MRK's existing vaccines have run into "carrier protein overload" (check appendix) that has blocked them from expanding to cover more serotypes. PCVX does not have the same platform limitation; VAX-31 uses half the carrier protein as conventional vaccines – enabling expanded serotype coverage. With VAX-31 covering 95% of IPD cases, this represents the last market shift in the PCV space and PCVX is best positioned to win out.

De-risked data, small caveat (1) VAX-31 read out pristine Ph1/2 (n=1015) Adults data in Sep 2024, advancing into Ph3 for Adults(OPUS program, reading out in Q4) and Ph2 for Infs. (2) VAX-24 read out imperfect Ph2 infant data in Mar 2025, missing on 5/24 serotypes (vs expected 3/24). Data still looks approvable, and coverage is better than PCV20 nonetheless (VAX24 missed on insignificant serotypes, accounting for negligible proportion of IPD cases). Moreover, a modification to look at a higher dose was made for VAX31 to avoid the same problems. This caused a sharp knee jerk reaction in the price and made PCVX available at a massive discount

Valuation

Peak sales greater than EV. PCVX should be able to take the majority of the global $6B+ PCV market. Improved coverage should allow for a price premium of 10-15%. Overall, consensus estimates peak sales at $6.2B. We estimate $5B US sales and ~7.5B worldwide sales (WW). Our $74.40 PT is reasonably conservative (US-only sales, 80% Ph3 PoS, 90% regulatory PoS, 10% discount, 3Y to market), likely has unmodelled upside in the WW opportunity.

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01 March 2026

PEPGEN INC (PEPG)

Buy | $6.22 | PT $17

Initiating Coverage

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Best-ever biomarker for DM1 likely to translate to best-in-class functional benefit.

With the PEPG 5mg/kg MAD data impending, PEPG has rallied significantly. Several well-known specialist funds have disclosed new/increased positions in 13Fs. Despite this, the FDA's move to put a partial clinical hold on the program has resurfaced concerns and affected the stock -18.57%. We argue that this hold is not a big deal and does not impact the drug in the long-run; (1) the hold is based on mice data that the FDA has had for over a year (2) the mice data is not relevant to humans (3) unrelated to kidney toxicity (3) does not affect the timeline of the 5 and 10mg/kg readouts, which proceed as planned and seems to be limited to the 15mg/kg cohort.

Investment Thesis

Company overview. PEPG is developing a treatment for Myotonic Dystrophy Type 1 (DM1) is a degenerative muscular disorder affects 40,000 patients in the US. In a Phase 1 clinical trial, PGN-EDODM1 has demonstrated the best splicing correction so far. If the effect is sustained (which looks likely to us), PEPG could produce best-in-class functional data to beat out competitors (Avidity/RNA/NVS 's del-desiran and DYN's DYN-251). Both are further ahead in clinical trials, and have shown some functional data in the key regulatory functional endpoint – vHOT.

Safety signal dragging on valuation . In our view, there are two factors which could be keeping the valuation back at this stage: (1) the data is very early stage (in fact, about as early as you can get) and, (2) some safety concerns. Re: (1) As we move through to MAD and then to Ph2 a more substantive dataset will take shape and this concern will clear away. Re: (2), the safety signal at the moment seems to be very limited –the creatinine elevations which had raised this concern were not above the upper limit of normal (ULN) – they were not a strong indicator of anything close to kidney/liver toxicity. More broadly, the PMO/PPMO class that PGN-EDODM1 is a part of does has had some tox issues in the past; this explains the sensitivity of the market to slight elevations in creatinine. We will watch safety closely going forward, and do some further digging on safety over the next quarter.

FDA's partial clinical hold follows multiple other PPMO holds over the last few years that were ultimately lifted. It is related to mice blood pressure drops driven by fast infustion; slow infusion seems to solve these problems and is unlikely to be a problem in humans. Moreover, 5mg and 10mg cohorts are not affected and will readout on time.

Valuation

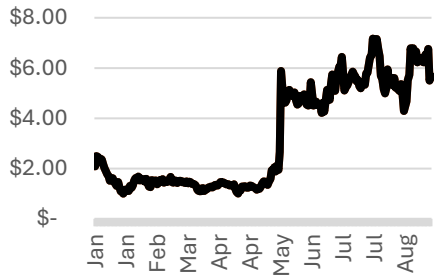
Peak sales in the range of a billion up for grabs. Both del-desiran and DYN-251 are valued at several billion dollars. We estimate peak sales to be approximately $1B (. The key assumption here is a 50% penetration, which we anchor on the 30-35% penetration in DMD patients amenable to exon skipping PMO therapies (which work very poorly, but have still achieved quite good penetration). Given the market potential, it is clear that PEPG is heavily discounted – either due to safety or the early nature of the data.

Key Data

Price	$	6.22
Shares (M)	$	69
MC (M)	$	430
Cash (M)	$	148
Debt (M)	$	19
EV (M)	$	300

Lead Asset	PGN-EDODM1
Phase	SAD / 1
Peak Sales ($B)	~1B
Price Target	$17
Catalyst	5g/kg MAD
Catalyst Date	1Q 2026

L12M Performance



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01 March 2026

SOLENO THERAPEUTICS (SLNO)

Buy | $39.07 PT $70

Update

Blockbuster rare disease launch

SLNO has remained at its highly discounted valuation throughout February. Sales have continued to outperform expectations and script data indicates a recovery of new patient starts in January. We continue to track this highly successful launch which is available at a dirt-cheap valuation.

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Investment Thesis

Overview SLNO is a commercial-stage company with VYKAT XR, the first and only FDA-approved treatment for hyperphagia in Prader-Willi syndrome (PWS) patients aged 4+.

Prader Willi Syndrome is a rare genetic disorder affecting ~10,000 diagnosed patients in the US, characterized by insatiable hunger (hyperphagia) leading to dangerous food-seeking behaviours, obesity, and early mortality (mean age of death ~29.5 years). It is important to note that PWS is a very deadly disease, with most not surviving past mid-to-late teens.

SLNO trades at a very low multiple following a sharp selloff driven by: (1) a short report released in Aug 25, (2) fluid retention and congestive heart failure risk, and (3) slower than expected patient uptake. These concerns seem to be (so far) mostly unreflected in the commercial data.

Strong launch trajectory SLNO's early sales have been strong, beating consensus in in every quarter since the launch. VYKAT XR is now highly likely to become a blockbuster (>$1B in sales) and has the potential to reach >$2B in its peak year. Despite this, there have been some concerns regarding weak patient start forms in the last print, which came in a little below expectations. Moreover, there are some indications that patients could be holding off on increasing dosages as they titrate up to the maintenance dose, although there is not enough data to tell.

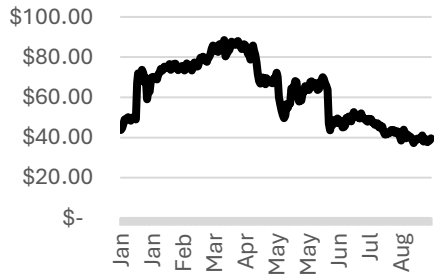
Valuation

Limited downside, big upside .Consensus places peak sales at $2.10B, which implies significant upside if fully realised (translates to around $85-126 per share at a 2-3x peak sales multiple). With the current sales velocity, it is almost guaranteed that VYKAT becomes a $1B+ drug. In this case, the stock will be worth $46.61-65.23 (at a 2x-3x multiple). Prior to the August short report, SLNO was trading in the $80-90 range.

Key Data

Price	$	39.07
Shares (M)	$	52
MC (M)	$	2,017
Cash (M)	$	305
Debt (M)	$	73
EV (M)	$	1,784
Lead Asset		VYKAT XR
Phase		Commercial
Peak Sales		$2.10B
Price Target		
Catalyst		Q1 earnings
Catalyst Date		April/May 2026

L12M Performance



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01 March 2026

VIR BIOTECHNOLOGY (VIR)

Buy | $9.09 | PT $22.86

Initiating Coverage

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Our top pick for the month – now a two asset biotech, greatly underappreciated

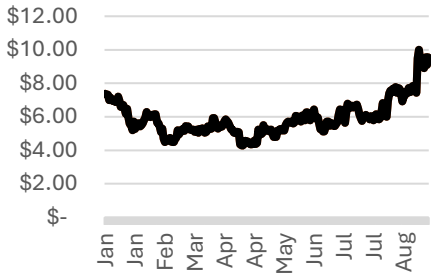
Last month, we highlighted VIR as one of our favourable stocks for their de-risked CHD program. In late Feb, VIR demonstrated that its secondary asset, VIR5500 and the masked TCE program is a heavyweight in its own respect – posting impressive data at ASCO GU in prostate cancer, beating out big pharma competitors. Astellas signed a deal with VIR for $1.7B dollars, and helping to take on 60% of the costs of the TCE program, in exchange for c.50% of the economics of the drug. Depsite this, VIR's stock has barely reacted, despite significant validation of the TCE program. With two shots on goal, a big pharma partnership, an ocean of cash and two scientifically important assets, we see VIR as our top pick for March. We have increased the position in anticipation of a re-rating over the medium term.

Key Data

Price	$	9.09
Shares (M)	$	160
MC (M)	$	1,452
Cash (M)	$	461
Debt (M)	$	98
EV (M)	$	1,089

Lead Asset	Tobevibart + elebsiran
Phase	2
Peak Sales ($B)	$2.34B
Price Target	
Catalyst	Further masked TCE data
Catalyst Date	

L12M Performance



Investment Thesis

Hepatitis D opportunity. VIR is developing a combo treatment of tobevibart (in-house mab) and elebsiran (siRNA licensed from Alnylam) for Chronic Hepatitis Delta (CHD). CHD is a concurrent infection of hepatitis D virus (HDV) in patients with hepatitis B (HBV); HDV is a satellite virus which needs HBV infection to replicate. Patients with CHD progress to liver failure within 5 years, and have 3x the liver cancer incidence of HBV – a much more aggressive timeline than CHB alone. In a Jan readout, VIR demonstrated 88% of participants had undetectable HDV RNA (TND) at 96 weeks of treatment. This compares favourably to the standard of care, Gilead's bulevirtide of 48% at week 96 – a significant improvement. At this point, it looks like VIR will become the standard of care upon market entry.

Masked TCE – VIR5500. In addition to the best-in-class CHD drug, VIR is a developing masked TCE for oncology indications. T cell engagers (TCE) aid the immune system (T cells) in targeting tumours by binding to both a T cell and a tumour cell(bispecific antibody), leading to cell lysis. This class is being investigated by a wide range of players, including a lot of interest from big pharma. **At ASCO GU, VIR presented standout data** – potent efficacy ORR of 45% and PSA 50/90 of 65% and 39% respectively in late-stage patients with a median of four prior lines of therapy. CRS was mild

Deal with Astellas The VIR5500 data has clearly impressed big pharma and VIR signed a deal with Astellas for **$1.7B** ($355M upfront, the rest in milestones) in exchange for **50%** of the US economics on VIR5500 and ex-US rights (VIR retains double digit royalties). 60% of the global development costs will be borne by Astellas as well, reducing cash burn for VIR. Overall, this deal could drive billions of dollars of value to VIR and validates the masked TCE program.

Valuation

Blockbuster potential in CHD. We reiterate our prior assessment of peak sales at $2.3B for the CHD program. Paired with an 80% PoS, and a 2x EV/Peak Sales multiple, we assign $13.42/ share to the CHD program. With a 20% PoS (highly conservative) assigned to the mCRPC program and $1.5B in peak sales, we value the remaining economics for the masked TCE program at $1.42 / share, the Astellas milestone payments (40% PoS) at $2.73 / share and the net cash at $5.29 / share. This brings our SOTP valuation to$22.86 / share.

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Closed Positions

01 March 2026

ETON PHARMACEUTICALS (ETON)

Closed | $16.99

Update

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Portfolio expansion and PDUFA completed

Eton Pharmaceuticals is developing a constellation of relatively simple therapies for various rare diseases. A unique market expansion strategy which can potentially expand its biggest product market by 5x caught our eye. Sustained revenue growth has made the company cash flow positive, and ETON could become profitable in 2026. The Q4 earnings call is scheduled for Mar 12 and will provide an update to the label harmonization expansion opportunity. We have recently closed the position with a profit thanks to the announced acquisition and PDUFA win.

Expansion plan

Roll-up strategy Eton has ten commercial products (KHINDIVI, INCRELEX, ALKINDI SPRINKLE, GALZIN, PKU GOLIKE, Carglumic Acid, Betaine Anhydrous, and Nitisinone, ET-600 and HEMANGEOL) across a range of rare diseases. ET-600 was recently approved for Diabetes Insipidus (characterised by large urine output and extreme thirst). ETON has also in-licensed HEMANGEOL, bringing in two new products this quarter. This portfolio has done well historically, delivering 19 straight quarters of revenue growth.

Pipeline ETON has four products in late-stage development (ET-700, ET-800, Amglidia, ZENEO). Overall, the pipeline has around ~$250M of peak sales potential across a range of diseases.

Market Expansion ETON is looking to expand the market of its two top products, INCRELEX and KHINDIVI. INCRELEX is indicated for SPIGFD (a rare disease characterised by impaired growth due to IGF-1 levels), but the exact definition of SPIGFD varies – the EU classified SPIGFD as IGF-1 levels more than 1.96 standard deviations (SD) away from normal levels, while the US defined it as 3 SD away from normal. ETON had a meeting with the FDA in December to discuss "harmonizing" these definitions. If FDA alignment is achieved, the patient population could grow from 200 to 1000, significantly increasing the revenue opportunity. Results of the meeting will be disclosed during the next earnings call, Mar 12.

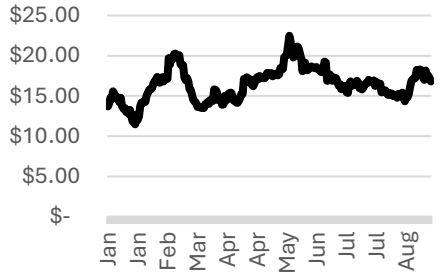
With KHINDIVI, the company is initiating a study in toddlers/infants < 5 years and plans to submit an NDA supplement in Q2 2026, for early 2027 approval. Furthermore, ET-700 (an extended release zinc medication for patients with Wilson disease, copper accumulation in the body) should read out some data mid 2026. Overall, we recommend a small position to track these expansion opportunities.

Key Data

Price	$	16.99
Shares (M)	$	27
MC (M)	$	456
Cash (M)	$	15
Debt (M)	$	30
EV (M)	$	471

Lead Asset	INCRELEX
Phase	Commercial
Portfolio Peak Sales	$500M
Price Target	
Catalyst	Label harmonization
Catalyst Date	12 Mar

L12M Performance



Valuation

Some opportunity left.With peak sales potential of ~200-275M across the approved product portfolio, the valuation of ~400M EV is reasonable. Label harmonisation can deliver ~100M incremental revenue expansion to the approved portfolio, and the pipeline accounts for another ~200-300M of peak sales potential. Overall, there is enough there to call ETON undervalued. However, we see better opportunities to deploy capital.

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01 March 2026

EYEPOINT PHARMA (EYPT)

Buy | $17.56

Update

Hedge worked, but sold too early..

Last month, we wrote that should OCUL run into trouble with their unconventional approach, EYPT should serve as a safe haven for investors wanting exposure to the TKI implant approach but fleeing from the OCUL-specific problems. Broadly, this trade has taken place with EYPT +23.00% from the Feb17 OCUL data release. Unfortunately, we made a mistake in exiting the position too early, spooked by the aflibercept overperformance resulting in the potential risk that EYPT will be unable to secure the intended non-inferiority label, therefore missing out on upside

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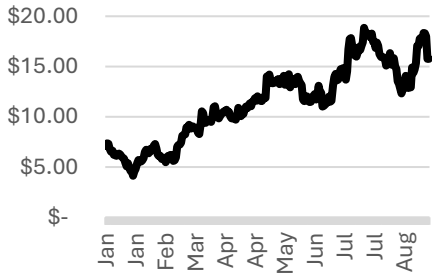
Key Data

Price	$	17.56
Shares (M)	$	83
MC (M)	$	1,465
Cash (M)	$	371
Debt (M)	$	23
EV (M)	$	1,117

Lead Asset	DURAVYU
Phase	3
Peak Sales	$1B
Price Target	
Catalyst	Ph3 readouts
Catalyst Date	1H 2026

L12M Performance



Investment Thesis

More data than Ocular, but slower DURAVYU is another intravitreal implant (very similar to AXPAXLI) which delivers a TKI (VEGF receptor inhibitor) over time, with injections occurring every 6 months. A key difference is that DURAVYU is actually shipped and stored at room temperature (it is a solid insert vs AXPALI gel), which could make it slightly easier to commercialise. EYPT sensibly decided not to brazenly skip Phase 2, so we have good comparator data. DAVIO 2, the Phase 2, demonstrated statistical non-inferiority in vision vs aflibercept Q8W. Note: This trial was run against the actual intended dosing frequency of aflibercept, one every 8 weeks and not against a single dose as is the case with OCUL's superiority trial design. DAVIO 2 showed 80%+ reduction in treatment burden, 64% of eyes supplement free. A little worse than OCUL nominally, but the difference is insignificant when accounting for (1) no responder enrichment (OCUL had required that patients achieve perfect 20/20 vision before enrolment) (2) full-on label dosing. We expect the actual treatment effect to be very similar if not identical between AXPAXLI and DURAVYU.

Two Phase 3 trials better comply with precedent. Historically, sponsors have had to run two large Phase 3 trials to secure approval in large indications such as wAMD. OCUL has taken advantage of recent comments by the FDA commissioner (link) that companies could pursue approval from a single trial. We take this with a grain of salt; the same commissioner recently bragged in SFO airport about relaxing regulations to approve more cell and gene therapies only to deny Atara's cell therapy approval the very next morning. EYPT, in contrast, has fully enrolled two Phase 3 studies, LUGANO and LUCIA to pursue non-inferiority to on-label EYLYEA (aflibercept). Not only is this a safer regulatory pathway, but the non-inferiority design is also more relevant to prescribers (as a direct comparison to on-label EYLEA). These trials are set to read out in 1H 2026 and are very likely to hit.

Valuation

PT of $28.27 from de-risked data. On the one hand, we model a lower peak sales of $1B as AXPAXLI could hit the market sooner than DURAVYU. However, we model 80% PoS for the Phase 3 trials (on account of strong Phase 2 data) and 90% regulatory PoS (on account of the two trial pathway) to produce a price target of $28.27 based on the same 3x multiple

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01 March 2026

LARIMAR THERAPEUTICS (LRMR)

Closed | $5.31

Initiating Coverage

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FDA Breakthrough Designation Secured. A successful trade, though we sold a little early

During our last coverage of Larimar, we stated "Regulatory clarity followed by BLA submission is imminent and we believe the equity to be favourably positioned for these catalysts."

Since then, nomlabofusp has received BTD (Breakthrough Therapy Designation) from the FDA, and re-iterated intent to submit BLA in June 2026. This trade has gone to plan, so we closed out our position for a profit. Admittedly, the stock price reaction was somewhat delayed and we sold the equity a little early. At the current price of $5.46 per share, we believe Larimar trades around fair value so we won't be coming back unless there is a substantial discount from here.

Investment Thesis

Friedrich's Ataxia. Endogenous frataxin is a protein that is imported to the mitochondria of a cell with a mitochondrial targeting sequence (MTS) that is synthesized alongside the full-length protein and cleaved by mitochondrial processing peptidase (MPP) inside the mitochondrion to yield mature human frataxin. Friedrich's Ataxia is a disease characterized by greatly depleted mitochondrial frataxin levels, which lead to a variety of symptoms including ataxia (for which the disease is named), cardiomyopathy and impaired neurological function. The pathway by which a deficiency in frataxin expression mediates these symptoms is unclear, but the relationship itself is undisputable. Importantly, skin biopsies from heterozygous carriers of the disease demonstrate impaired frataxin expression (c60% of normal levels), yet these carriers are (almost) asymptomatic. Biopsies from homozygous sufferers of the disease show much lower levels (c20%) and this population is of course symptomatic. These findings substantiate the thesis that a deficiency in frataxin correlates to disease severity in a non-linear way.

Larimar's treatment approach is simple and consists of introducing frataxin in a protein replacement therapy (nomlabofusp) via intravenous infusion of the MTS + full length protein attached to a cell-penetrating peptide (CPP) which is attached directly onto the MTS allowing both targeting peptides to be cleaved by MPP once inside the mitochondrion.

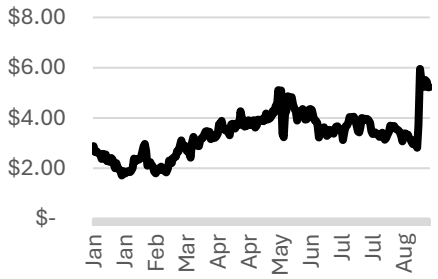
Anaphylaxis. The drug has a high incidence of anaphylaxis but patients become accustomed to it over time, and are provided with epipens by the company. The rate of mortality from adequately controlled anaphylaxis is almost zero and we don't believe that this will preclude the drug from approval given the severity of the indication and the well-known methods for controlling such AEs.

Key Data

Price	$	5.31
Shares (M)	$	104
MC (M)	$	550
Cash (M)	$	183
Debt (M)	$	5
EV (M)	$	372

Lead Asset	nomlabofusp
Phase	BLA filing
Peak Sales	
Price Target	
Catalyst	
Catalyst Date	

L12M Performance



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01 March 2026

OCULAR THERAPEUTIX (OCUL)

Closed | $8.94

Update

SOL-1 unsuccessful, insufficient for commercial success.

SOL-1 showed statistical success, but the difference between drug and control arm was smaller than people anticipated. The benefit of AXPAXLI slightly underperformed projections whilst single dose aflibercept 2mg outperformed expectations *(due to lack of data on this unconventional single dose protocol)*. The narrow difference will make the commercial situation significantly more challenging, and we model half the penetration as well as 20% lower PoS than in our prior model. These factors reduce the PT to around $9.50, and we made the decision to close the position following the negative study results.

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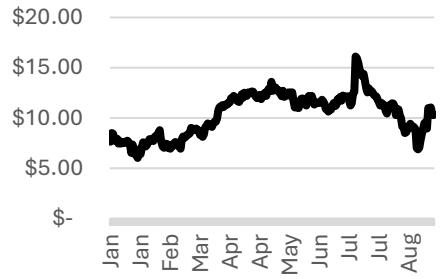
Key Data

Price	$	8.94
Shares (M)	$	218
MC (M)	$	1,946
Cash (M)	$	737
Debt (M)	$	77
EV (M)	$	1,286

Lead Asset	AXPAXLI
Phase	3
Peak Sales	$1.00B
Price Target	$9.50
Catalyst	
Catalyst Date	

L12M Performance



Investment Thesis

Company overview The wet/neovascular (wAMD) market has been plagued with low adherence rates due to the invasive nature of the standard of care. wAMD is caused by abnormal blood vessel growth (neovascularisation) beneath the retina driven by VEGF (vascular endothelial growth factor, which signals for angiogenesis) overexpression, triggered by age-related changes in the macula that lead to tissue hypoxia. The established standard of care, EYLEA, VABYSMO, PAVBLU inhibit VEGF and stop the neovascularisation very effectively. Problematically, they have to be administered by intravitreal injections (into the eye) multiple times per year. The unpleasant administration route causes about ~40% of patients to discontinue over the long term or skip doses, despite the strong efficacy profile. OCUL (along with competitor EYPT) places a tiny gel implant which blocks VEGF binding by releasing small amounts of drug over time, enabling just 2 injections per year.

SOL-1 results showed a underwhelming improvement in efficacy over the comparator, single dose aflibercept 2mg. 74% of Axpaxli –treated patienbts maintained visual acuity vs 56% aflibercept-treated (an 18% delta). Experts and analysts were looking towards a delta closer to 30% for an upside scenario. There were minor safety concerns, generally judged to be acceptable by KOLs. **Our view** is that this data is insufficient to see a commercially competitive label; **(1)** the control arm in this trial is typically dosed one every two months, not a single dose – setting a low bar **(2)** aflibercept 2mg is being succeeded by aflibercept 8mg (EYLEA HD) and faricimab (VABYSMO) which would further degrade the delta **(3)** the superiority label to single-dose aflibercept will not be favourably received by physicians and (4) the single-trial approval pathway is very new and so far untested; the FDA has shown it follows its guidance loosely, and this thus creates additional risk.

On the other hand, SOL-1 was statistically significant and was nominally successful on every metric, they will likely secure approval.

Valuation

Uncertain commercial landscape We revise our penetration parameter to half of our pre-SOL-1 value, which reduces peak sales estimates to $1B. At the same time, we reduce by 20% the regulatory PoS from 80% (conditional on Ph3 successs) to 60% as we feel that the regulator will raise questions regarding the superiority of AXPAXLI to on-label aflibercept.

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01 March 2026

TECTONIC THERAPEUTIC (TECX)

Closed| $23.45

Initiating Coverage

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AZ3426 discontinued, waiting for clarity on relaxin approach. Rebound and investor interest suggest more to the story.

Last month we initiated a position in TECX ,looking for readthrough from AZN's relaxin program which has two molecules, AZD3427 (SQ) and AZD5462 (oral). In a pipeline update on Feb 10, AZN discontinued AZD3427 due to "efficacy" (that's all she wrote) whilst moving forward with AZD5462, creating a mixed signal. With the trial populations much more similar to TECX's APEX trial, focus has now shifted to differentiation based on the molecular differences between AZD3427. We are currently waiting on (1) the **completion of a private laboratory** study on the differences between the two molecules and (2) the publication of full data (paper currently being written, per sources) AZD3427 (similar to the volenrelaxin timeline). NB: despite the discontinuation being negative news, TECX has rebounded to $23 (above both our entry and exit). Note increased hedge fund (HF) interest even post-discontinuation (per sources). Moreover, Baker Bros (the pre-eminent biotech HF) increasing their position 9x in Q4 2025.

Investment Thesis

Company overview . TECX is developing TX45, an extended half life RXFP1 agonist, for Pulmonary Hypertension Heart Failure with preserved Ejection Fraction (PH-HFpEF). Put simply: the patient's heart has become stiff (LV dysfunction) and is unable to fill properly (this is the HFpEF part). The blood coming into the left ventricle (LV) has no room and more remains in the lungs, causing high pressure in the lungs (Pulmonary Hypertension, PH). There are two subtypes: ipcPH-HFpEF(isolated post capillary PH-HFpEF) and cpcPH (combined pre and post capillary PH). cpcPH happens when the excess pressure in the lungs damages the blood vessels and causes scarring (pulmonary vascular disease), constricting them. This causes more pressure on the right side of the heart and results in right heart failure in addition to left heart failure – cpcPH is more severe as a result of the vascular remodelling.

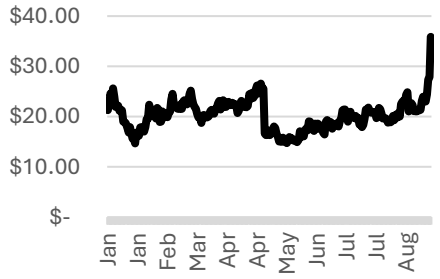
Relaxin and TX45 During pregnancy, the body's cardiac output (CO) increases by 40-50%. This would normally cause heart failure, but the body releases a hormone, relaxin, to accommodate. Relaxin has many effects and acts through many mechanisms: vasodilation (through cAMP and cGMP), cardiomyocyte survival (through MAPK), preventing collagen deposition & encouraging degradation (and therefore fibrosis/scarring, through TGF-beta inhibition and MMP upregulation). Overall, it is a vasodilator hormone that decreases the pressure (in a targeted manner) and has a remodelling effect (through the anti-fibrotic properties) that helps with the vascular disease in the lungs. **TX45** binds to the relaxin receptor (RXFP1) and triggers these effects, mostly in the kidneys and vasculature. Earlier relaxin drugs have been tried and they have failed, but they were misdirected. TECX is running a trial in cpcPH, a population with the most to gain from the remodelling and vasodilative effects. Moreover, the molecule is pharmacokinetically better designed than its predecessors.

Key Data

Price	$	23.45
Shares (M)	$	19
MC (M)	$	440
Cash (M)	$	254
Debt (M)	$	3
EV (M)	$	190

Lead Asset	TX45
Phase	2 / Pivotal
Peak Sales ($B)	$3.00B
Price Target	$50
Catalyst	Mar/April 2026
Catalyst Date	AZ 3427 full data publication

L12M Performance



Valuation

Holy grail indication. 1.4M people suffer from PH-HFpEF and 400k suffer from cpcPH - with no treatments. Median survival is less than 3 years, which is worse than some cancers. A treatment in this indication would be incredibly impactful and highly valuable. A real shot on goal is very worthwhile paying ~100M in EV.

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01 March 2026

UNIQURE NV (QURE)

Closed| $15.63

Update

Complete FDA pullback has made the position untenable.

QURE has the potential to market a very impactful therapy for Huntington's Disease – a functional cure. However, recent leadership changes at the FDA (Vinay Prasad) have lead the FDA to renege on their previous guidance. The FDA has decided to stick to an uncompromising hard-line and requiring an immoral, lengthy (likely a 3 year) sham-controlled trial (for which it will be very difficult to find patients). This has created a delay of five years, a massive cost, and heartbreak for HD patients.

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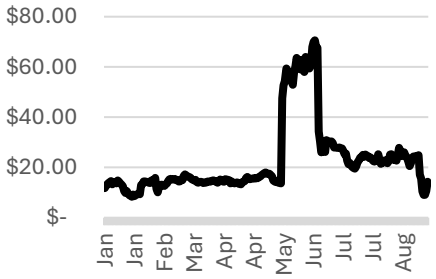
Key Data

Price	$	15.63
Shares (M)	$	63
MC (M)	$	977
Cash (M)	$	623
Debt (M)	$	76
EV (M)	$	431

Lead Asset	AMT-130
Phase	Ph2/3
Total Revenue Opportunity($B)	$20B
Price Target	
Catalyst	
Catalyst Date	

L12M Performance



Investment Thesis

A "cure" for Huntington's Disease? In September, QURE reported that their AAV gene therapy, AMT-130 had slowed progression of Huntington's Disease (HD) by 75%, as measured by cUHDRS at three years. With the onset of HD happening at 35-40 years old, extending the survival time from 15 to 45 years would effectively give HD patients a normal life expectancy. This attracted a great deal of generalist interest and saw the company gain 240%+ in a single day, significantly more than institutional investors expected (based on investor surveys prior to data).

The gene therapy works by delivering AAV directly into the striatum (a deep region of the forebrain) by live MRI-guided stereotactic neurosurgery to reduce huntingtin production (HTT). In reducing HTT, production of both mutant HTT (mHTT, the primary disease driver of HD) and normal / wild-type HTT is decreased – resulting in slowing of disease progression.

The regulatory saga is lengthy. Recall, following the Sep 2025 data mentioned above, QURE met with the FDA in Nov 2025 and disclosed that the FDA may no longer be aligned on the regulatory pathway; i.e. the FDA decided that they don't want to stick to their prior guidance that the historical control study design would be sufficient for approval. QURE scheduled a type A meeting with the FDA for Jan 9th and the meeting minutes which were released on Mar 02 confirmed that the FDA will require a randomized, sham-controlled study. This would mean that QURE would have to enrol patients for a trial in which sham neurosurgery would be performed, thus creating significant difficulties with enrolment. Moreover, the HD trials are very long and this therefore represents a 4 year delay. As a result, QURE stock has sold off to $9. Fortunately, we were to close the position at $17, mitigating some of the loss.

Valuation

All eyes on FDA. HD has around 40,000 patients in the US. AMT-130 delivers at least 15 QALYs and should confidently secure pricing of $2-3M. Using a discounted risk-adjusted cumulative sales multiple, we value QURE at $20.73/ share (20% penetration, 50% Ph3 PoS, 60% regulatory PoS, 24% discount rate, 5Y to market). The expensive trial now required by the FDA means that QURE will likely need to secure an additional $1B (or more) to run the trial and sustain cash burn over another five years. Adjusting for this, we place FV at $7-10, depending on the cost of trial and the terms of the financing over the next few years. To summarise, the decrease in Ph3 PoS, regulatory PoS, an increase in the discount rate (c.13% → 24%) and the need for additional financing has hammered the value of the stock, and warrants the liquidation of the position.

01 March 2026

DIANTHUS THERAPEUTICS (DNTH)

Closed | $55.19

Initiating Coverage

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Strong differentiation vs SoC on safety and dosing will enable claseprubart to step into a large rare disease market.

After a conversation with management in late Feb we opened a position in DNTH as we felt that it had a clear differentiation in safety, more convenient dosing and strong Ph2 data. Moreover, the market expansion opportunities beyond gMG in CIDP and MMN provide good upside in peak sales potential as the data in those indications further matures. We were able to realise a significant profit without taking on much risk and decided to close the position during the ~20% rally in late February.

Company overview

Company overview. Dianthus is developing claseprubart, an active C1s antibody , for rare and severe autoimmune diseases, with the lead indication being generalised Myasthenia Gravis (gMG)

Generalised Myasthenia Gravis (gMG) is an autoimmune condition affecting approximately 100,000 patients, wherein autoantibodies attack the nicotinic acetylcholine receptor (AChR) causing membrane destruction in the joints and muscles.

Standard of care (SoC) several C5 treatments have been approved for this indication (VYVGART, RYSTIGGO, ZILBRYSQ, ULTOMIRIS). These stop the complement system from forming the membrane attack complex (MAC) once an autoantibody binds to the cell.

Claseprubart would be the first treatment without a boxed warning . DNTH's drug only stops the classical pathway , meaning that two alternative pathways are still available for fighting off severe infections whilst the key pathway driving gMG is inhibited. This is a key differentiator for physicians, as it would have comparable efficacy without carrying the risk of severe complications.

Dosing convenience. A more convenient administration route (autoinjector vs pre-filled syringe) and more infrequent dosing vs SoC would serve to further differentiate claseprubart.

Other indications DNTH is also pursuing two other (rarer) diseases – Chronic Inflammatory Demyelinating Polyneuropathy (CIDP) and Multifocal Motor Neuropathy (MMN) where there is slightly less competition. They serve as excellent adjacent indications and can significantly expand the sales potential of claseprubart.

Valuation

Existing treatments have already developed a large market for DNTH to step into. C5s treatments have an estimated ~2B market. Thanks to strong differentiation, clasprubart can take significant market share away from this class. Moreover, the more convenient and less frequent dosing can enable claseprubart to further grow the total sales within the indication.

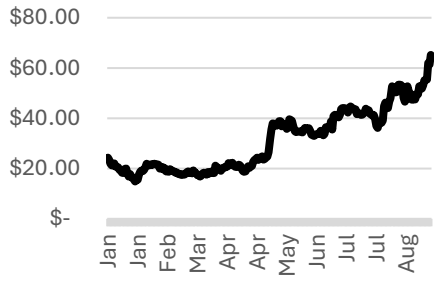
Key Data

Price	$	55.19
Shares (M)	$	43
MC (M)	$	2,366
Cash (M)	$	275
Debt (M)	$	1
EV (M)	$	2,093

Profile

Lead Asset	Claseprubart
Phase	Ph3
Peak Sales ($B)	>$1B
Price Target	N/A
Catalyst	CAPTIVATE Ph3 in CIDP topline
Catalyst Date	Q2 2026

L12M Performance



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Appendix

Recap of drug and market NKTR's rezpegaldesleukin produced some pivotal data in 2025 which made it a very strong option for

MAD study (Multiple Ascending Dose): A type of early clinical trial where patients receive multiple doses of a drug at increasing levels to test safety and how the body processes the drug over time.
Biomarker: A measurable biological indicator (like a protein level or gene expression) that shows whether a drug is having its intended effect in the body.
Functional data: Clinical trial results measuring real-world patient improvements (like ability to walk or grip strength) rather than just laboratory measurements.
Endpoint: The specific outcome a clinical trial is designed to measure to determine if a treatment works.
Sham: A fake procedure used as a placebo in surgical or device trials, where patients undergo a similar experience without receiving the actual treatment.
Open label trial: A clinical trial where both doctors and patients know who is receiving the treatment (no placebo blinding).
Phase 1/2/3: Sequential stages of drug testing. Phase 1 tests safety in small groups; Phase 2 tests effectiveness; Phase 3 confirms results in large populations before approval.
Superiority trial: A trial designed to prove one treatment is better than another, not just equivalent.
Non-inferiority: A trial designed to show a new treatment is at least as good as an existing one (not worse).
Responder enrichment: Selecting trial participants who already show signs of responding well to treatment, which can make results look better than they would in a general population.
Regulatory Terms
PDUFA (Prescription Drug User Fee Act): The deadline date by which the FDA must make a decision on a drug application.
Type A meeting: An urgent FDA meeting to resolve critical issues blocking drug development (typically scheduled within 30 days).
Type B meeting: A standard FDA meeting to discuss drug development strategy (typically scheduled within 60 days).
BLA (Biologics License Application): The formal application submitted to FDA to request approval for a biological drug.
NDA (New Drug Application): The formal application submitted to FDA to request approval for a conventional drug.
Natural history control: Using data from untreated patients (collected separately) as the comparison group instead of a placebo arm in the same trial.
Drug & Biology Terms
Orphan disease: A rare disease affecting fewer than 200,000 people in the US, qualifying drugs for special incentives.
Blockbuster: A drug generating over $1 billion in annual sales.
Splicing correction: Fixing errors in how genes are read and processed into proteins.
vHOT (video Hand Opening Time): A functional test measuring how quickly patients can open their hand, used in muscular dystrophy trials.
Cardiac myosin inhibitor: A drug that reduces the force of heart muscle contractions by targeting myosin (the protein that makes muscles contract).
TCE (T-Cell Engager): An antibody that grabs both a T-cell (immune cell) and a target cell (like cancer), forcing the immune system to attack the target.
Masked TCE: A T-cell engager designed to remain inactive until it reaches the target tissue, reducing side effects elsewhere in the body.
mab (monoclonal antibody): A lab-made protein designed to bind to a specific target in the body.
siRNA: Small interfering RNA; a molecule that silences specific genes by destroying their messenger RNA.
AAV (Adeno-Associated Virus): A harmless virus used to deliver gene therapies into cells.
Autologous gene therapy: A treatment using the patient's own cells, which are modified and returned to their body.
Disease-Specific Terms
Friedrich's Ataxia (FA): A genetic disease causing progressive nerve damage, loss of coordination, and heart problems due to lack of frataxin protein.
Frataxin: A mitochondrial protein essential for cellular energy production; its deficiency causes Friedrich's Ataxia.
Myotonic Dystrophy Type 1 (DM1): A genetic muscle disease causing progressive weakness, prolonged muscle contractions, and multiple organ problems.
PH-HFpEF (Pulmonary Hypertension with Heart Failure with preserved Ejection Fraction): High blood pressure in the lungs occurring in patients whose heart pumps normally but fills poorly.
cpcPH (combined pre and post capillary Pulmonary Hypertension): A severe form of PH-HFpEF where lung blood vessels become scarred and constricted.
ipcPH-HFpEF (isolated post capillary PH-HFpEF): A milder form where lung pressure is elevated but vessels aren't yet damaged.
HCM (Hypertrophic Cardiomyopathy): A condition where heart muscle becomes abnormally thick, making it harder for the heart to pump.
oHCM/nHCM: Obstructive vs non-obstructive HCM; whether the thickened muscle blocks blood flow out of the heart.
wAMD (wet Age-related Macular Degeneration): A leading cause of blindness where abnormal blood vessels grow under the retina and leak.
Ulcerative Colitis (UC): An inflammatory bowel disease causing ulcers and inflammation in the colon.
Crohn's Disease: An inflammatory bowel disease that can affect any part of the digestive tract.

Appendix

SLE (Systemic Lupus Erythematosus): An autoimmune disease where the immune system attacks multiple organs.
Rheumatoid arthritis (RA): An autoimmune disease causing joint inflammation and damage.
Huntington's Disease (HD): A fatal genetic brain disease causing progressive movement, cognitive, and psychiatric problems.
Prader-Willi Syndrome (PWS): A genetic disorder causing constant hunger, obesity, and developmental delays.
Hyperphagia: Insatiable, uncontrollable hunger.
Alopecia Areata (AA): An autoimmune condition causing patchy hair loss.
Atopic Dermatitis (AD): Chronic inflammatory skin condition (eczema).
Chronic Hepatitis Delta (CHD): A severe liver infection caused by hepatitis D virus co-infecting hepatitis B patients.
Invasive pneumococcal disease (IPD): Serious infections (pneumonia, meningitis, bloodstream infections) caused by Streptococcus pneumoniae bacteria.
RDEB (Recessive Dystrophic Epidermolysis Bullosa): A rare genetic skin disease where skin is extremely fragile and blisters easily due to missing collagen-7.
Hypoglycaemia: Dangerously low blood sugar.
Congenital hyperinsulinism (cHI): A rare genetic condition where the pancreas produces too much insulin, causing dangerous low blood sugar.
Tumour hyperinsulinism (tHI): Excess insulin production caused by a tumor (usually pancreatic).
Biological Mechanisms
Vascular remodelling: Changes in blood vessel structure, including scarring and constriction.
Hemodynamic measures: Measurements of blood flow and pressure in the heart and blood vessels.
LV dysfunction (Left Ventricular dysfunction): Impaired function of the heart's main pumping chamber.
Diastolic dysfunction: The heart muscle is too stiff to fill properly between beats.
LVOT gradient: The pressure difference across the narrowed outflow tract in obstructive HCM.
Cardiac output (CO): The volume of blood the heart pumps per minute.
Contractility: The force with which heart muscle contracts.
Neovascularisation: Growth of new blood vessels (abnormal in diseases like wAMD).
Fibrosis: Scarring of tissue.
cAMP/cGMP: Cellular signaling molecules that cause blood vessel relaxation.
MAPK: A signaling pathway involved in cell survival and growth.
TGF-beta inhibition: Blocking a protein that promotes scarring and fibrosis.
MMP upregulation: Increasing enzymes that break down scar tissue.
Cardiomyocyte: Heart muscle cell.
Mitochondria: The "power plants" of cells that produce energy.
Mitochondrial targeting sequence (MTS): A molecular "address label" that directs proteins into mitochondria.
Mitochondrial processing peptidase (MPP): An enzyme inside mitochondria that clips off targeting sequences from arriving proteins.
Cell-penetrating peptide (CPP): A short sequence that helps large molecules enter cells.
Protein replacement therapy: Treating disease by supplying a missing or deficient protein.
Collagen-7 (COL7): A protein that anchors skin layers together; missing in RDEB.
Endogenous: Naturally produced by the body.
Heterozygous carriers: People with one normal and one mutant copy of a gene (often unaffected or mildly affected).
Anaphylaxis: A severe, potentially life-threatening allergic reaction.
B cell / T cell: Types of immune cells; B cells make antibodies, T cells directly attack threats.
Plasma cells: Mature B cells that produce large amounts of antibodies.
Regulatory T cells (Tregs): Immune cells that suppress excessive immune responses.
CD3/CD19/CD20: Proteins on the surface of immune cells used as drug targets. CD3 is on T cells; CD19 and CD20 are on B cells.
CAR-T therapy: Genetically engineering a patient's T cells to attack specific targets (usually cancer).
Bispecific antibody: An antibody engineered to bind two different targets simultaneously.
Depletion data: Results showing how effectively a treatment eliminates target cells.
Autoreactive: Immune cells that mistakenly attack the body's own tissues.
CRS (Cytokine Release Syndrome): A potentially dangerous inflammatory response when immune cells are activated too strongly.

Appendix

PSA50: A 50% reduction in PSA (prostate-specific antigen), used to measure prostate cancer treatment response.
miR-124: A microRNA molecule that regulates inflammation.
Cytokines (IL-23, IL-6, IL-17): Signaling proteins that drive inflammation.
JAK inhibitors: Drugs blocking JAK enzymes involved in inflammatory signaling.
S1P1 modulators: Drugs affecting sphingosine-1-phosphate receptors, used in autoimmune diseases.
TL1A drugs: Drugs targeting TNF-like ligand 1A, a driver of inflammatory bowel disease.
Black box warning: The FDA's most serious warning label, indicating significant safety risks.
Second line treatment: Therapy used after the first-choice treatment fails or isn't tolerated.
Satellite virus: A virus that requires another virus to replicate (HDV needs HBV).
TND (Target Not Detected): Complete elimination of detectable virus.
Striatum: A deep brain region involved in movement control.
Stereotactic neurosurgery: Precise brain surgery using 3D coordinates.
Huntingtin (HTT): The protein whose mutant form causes Huntington's Disease.
mHTT (mutant huntingtin): The abnormal protein causing HD.
cUHDRS: Composite Unified Huntington's Disease Rating Scale; measures HD progression.
QALY (Quality-Adjusted Life Year): A measure of disease burden including both quantity and quality of life.
Pharmacology Terms
Pharmacokinetic (PK) profile: How the body absorbs, distributes, metabolizes, and eliminates a drug.
Therapeutic window: The dose range where a drug is effective without being toxic.
Upper limit of normal (ULN): The highest value considered normal for a lab test.
Creatinine elevations: Increases in a kidney function marker that may indicate kidney stress.
PMO/PPMO: Types of antisense molecules (morpholinos) used to modify gene expression.
Half-life: How long it takes for half of a drug to be eliminated from the body.
Treatment interruption: Temporarily stopping a drug due to side effects or safety concerns.
Penetration: The percentage of eligible patients who actually receive a treatment.
Serotype coverage: The number of bacterial strains a vaccine protects against.
Carrier protein overload: When too much carrier protein in a vaccine prevents the immune system from responding properly to all antigens.
ACIP recommendations: Vaccination guidelines from the CDC's Advisory Committee on Immunization Practices.
Business & Valuation Terms
EV (Enterprise Value): Company value calculated as market cap plus debt minus cash; represents what it would cost to buy the whole company.
MC (Market Cap): Total value of a company's shares (share price × shares outstanding).
Peak sales: The highest annual revenue a drug is expected to generate.
rNPV (risk-adjusted Net Present Value): A valuation method that discounts future cash flows by both time and probability of success.
DCF (Discounted Cash Flow): Valuation based on projected future cash flows discounted to present value.
PoS (Probability of Success): The estimated chance a drug will succeed in trials and reach approval.
M&A (Mergers & Acquisitions): Companies buying or merging with other companies.
BO (Buyout): One company purchasing another.
Loss of exclusivity: When patents expire and generic/biosimilar competition can enter.
COE (Center of Excellence): A specialized medical facility with particular expertise in a disease or treatment.
Continuous glucose monitoring (CGM): A device that tracks blood sugar levels continuously.
Glucose infusion rate (GIR): How much glucose must be given intravenously to maintain normal blood sugar (used in hyperinsulinism studies).
Hawthorne effect: When study participants change their behavior because they know they're being observed.
Post hoc analysis: Analysis performed after a trial ends, looking at data in ways not originally planned (generally viewed skeptically).
Intravitreal injection: Injection directly into the eye.
BCVA (Best Corrected Visual Acuity): Vision measurement using optimal glasses/contacts.

Appendix

CIDP (Chronic Inflammatory Demyelinating Polyneuropathy): A rare autoimmune disorder in which the immune system attacks the myelin sheath surrounding peripheral nerves, causing progressive weakness and impaired sensory function in the legs and arms.
MMN (Multifocal Motor Neuropathy): A rare immune-mediated neuropathy characterised by progressive, asymmetric weakness without sensory loss, caused by autoantibodies attacking motor nerves.
gMG (Generalised Myasthenia Gravis): An autoimmune condition affecting approximately 100,000 patients in the US, wherein autoantibodies attack the nicotinic acetylcholine receptor (AChR) at the neuromuscular junction, causing muscle weakness throughout the body.
ATTR-CM (Transthyretin Amyloid Cardiomyopathy): A progressive heart disease caused by misfolded transthyretin (TTR) protein deposits in the heart muscle, leading to heart stiffness and heart failure.
AATD (Alpha-1 Antitrypsin Deficiency): A genetic disorder in which the liver produces insufficient or dysfunctional alpha-1 antitrypsin protein, leading to progressive lung disease (emphysema) and liver damage.
Achondroplasia: The most common form of short-limbed dwarfism, caused by a gain-of-function mutation in the FGFR3 gene that impairs bone growth at the growth plates.
LGMD2I (Limb-Girdle Muscular Dystrophy Type 2I): A rare inherited muscular dystrophy caused by mutations in the FKRP gene, leading to progressive weakness of the shoulder and hip girdle muscles.
Bronchiectasis: A chronic lung condition in which the airways become permanently widened and damaged, leading to mucus buildup, recurrent infections, and progressive lung function decline. Non-cystic fibrosis bronchiectasis (NCFB) refers to cases not caused by cystic fibrosis.
Kaposi's Sarcoma: A cancer that forms in the lining of blood and lymph vessels, often presenting as lesions on the skin, mouth, or internal organs; associated with immunosuppression and human herpesvirus 8 (HHV-8) infection.
ex20ins NSCLC (Exon 20 Insertion Non-Small Cell Lung Cancer): A subset of NSCLC driven by insertion mutations in exon 20 of the EGFR gene, historically resistant to standard EGFR-targeted therapies and requiring specialised inhibitors (e.g. zipalertinib).
AML (Acute Myeloid Leukaemia): An aggressive blood cancer characterised by rapid proliferation of abnormal myeloid cells in the bone marrow, impairing normal blood cell production.
Sickling: The process by which red blood cells deform into a rigid, crescent (sickle) shape due to polymerisation of abnormal haemoglobin S (HbS), obstructing blood flow and causing tissue damage.
VOC (Vaso-Occlusive Crisis): Acute episodes in sickle cell disease where sickled red blood cells block small blood vessels, causing severe pain, tissue ischaemia, and organ damage; the primary driver of morbidity and mortality in SCD.
Chronic Hemolytic Anaemia: A persistent condition in which red blood cells are destroyed (haemolysed) faster than they can be produced, leading to anaemia, fatigue, and progressive organ damage over time.
Classical Pathway: One of three activation pathways of the complement immune system, triggered when antibodies bind to targets on cell surfaces. Inhibiting only this pathway (as claseprubart does) leaves the alternative and lectin pathways intact for infection defence.
AChR (Nicotinic Acetylcholine Receptor): A receptor at the neuromuscular junction that binds acetylcholine to trigger muscle contraction; the primary target of autoantibodies in myasthenia gravis.
MAC (Membrane Attack Complex): The terminal product of complement activation (C5b-9) that forms pores in cell membranes, causing cell destruction. C5 inhibitors (e.g. eculizumab, ravulizumab) block MAC formation.
CRS (Cytokine Release Syndrome): A systemic inflammatory response triggered by massive cytokine release, typically occurring after T-cell engaging therapies; ranges from mild fever to life-threatening multi-organ dysfunction.
ORR (Overall Response Rate): The proportion of patients in a clinical trial whose tumour shrinks or disappears following treatment; includes both complete and partial responses.
mPFS (Median Progression-Free Survival): The length of time during and after treatment at which half of patients have not experienced disease progression or death.
PSA (Prostate-Specific Antigen): A protein produced by prostate cells; elevated levels in the blood are used as a biomarker for prostate cancer disease burden and treatment response.
PSA50: A 50% or greater reduction in PSA from baseline, used as a measure of treatment response in prostate cancer trials.
PSA90: A 90% or greater reduction in PSA from baseline, representing a deeper treatment response and generally associated with better clinical outcomes.
ASCO GU (American Society of Clinical Oncology Genitourinary Cancers Symposium): A major annual oncology conference focused on cancers of the urinary tract and male reproductive system, where key clinical trial data is presented.

Appendix

Masked TCE (Masked T-Cell Engager): A T-cell engager designed with a masking moiety that keeps it inactive in healthy tissue; the mask is cleaved in the tumour microenvironment, activating the drug locally to reduce systemic toxicity.
Bispecific Antibody: An engineered antibody capable of binding two different molecular targets simultaneously, enabling novel therapeutic mechanisms such as bridging immune cells to tumour cells.
Bispecific T-Cell Engager (BiTE / TCE): A bispecific antibody that binds CD3 on T cells and a tumour-associated antigen (e.g. PSMA, CD19), physically redirecting endogenous T cells to kill target cells.
Cell Lysis: The destruction of a cell by rupturing its membrane, releasing its contents; the intended outcome when T-cell engagers redirect immune cells against tumour or autoreactive B cells.
KRAS: An oncogene encoding the KRAS protein, a GTPase in the RAS-MAPK signalling pathway. KRAS mutations (found in ~25% of all cancers and ~90% of PDAC) lock the protein in an active state, driving uncontrolled cell proliferation.
KRAS G12C: A specific KRAS mutation (glycine to cysteine substitution at position 12) found in ~13% of NSCLC and ~3% of CRC; targetable by covalent inhibitors (e.g. sotorasib, adagrasib, zoldonrasib) that bind the mutant cysteine residue.
TKI (Tyrosine Kinase Inhibitor): A class of drugs that block tyrosine kinases — enzymes that activate signalling pathways controlling cell growth and survival. Used across oncology (e.g. CML, NSCLC) and ophthalmology (VEGF receptor inhibition).
siRNA (Small Interfering RNA): A short double-stranded RNA molecule that silences specific genes by binding to and triggering the degradation of their messenger RNA, preventing protein production.
TTR Stabiliser: A drug (e.g. tafamidis, acoramidis) that binds to and stabilises the transthyretin tetramer, preventing it from dissociating into monomers that misfold and aggregate as amyloid deposits in the heart.
PK Activator (Pyruvate Kinase Activator): A drug (e.g. mitapivat) that activates the red blood cell enzyme pyruvate kinase, restoring normal energy metabolism and reducing haemolysis in conditions like PK deficiency, thalassaemia, and sickle cell disease.
HbF (Fetal Haemoglobin): A form of haemoglobin normally predominant in foetal life that has higher oxygen affinity than adult haemoglobin (HbA). In sickle cell disease, HbF inhibits the polymerisation of HbS and prevents sickling; drugs inducing HbF (e.g. hydroxyurea, pociredir) are disease-modifying.
DPP1 Inhibitor (Dipeptidyl Peptidase 1 Inhibitor): A drug (e.g. brensocatib/BRINSUPRI) that blocks DPP1, a protease responsible for activating neutrophil serine proteases. By reducing neutrophil-driven inflammation, DPP1 inhibitors target the underlying pathology of bronchiectasis.
Prodrug: A pharmacologically inactive compound that is converted into its active form after administration, typically through metabolic processes. TPIP is a prodrug of treprostinil that slowly releases the active drug to enable once-daily dosing.
Refractory: Disease that does not respond to standard treatment, or disease that initially responds but subsequently stops responding.
Natural History: The typical progression of a disease over time in the absence of treatment; natural history data is used to model expected outcomes and can serve as an external comparator in clinical trials.
Intracellular cAMP (Cyclic Adenosine Monophosphate): A second messenger molecule generated by adenylyl cyclase activation; mediates downstream signalling including vasodilation in the prostacyclin pathway.
PKA-Mediated Phosphorylation: The process by which protein kinase A (PKA), activated by cAMP, adds phosphate groups to target proteins, triggering smooth muscle relaxation and vasodilation in the pulmonary vasculature.
MAPK Pathway (Mitogen-Activated Protein Kinase Pathway): A key intracellular signalling cascade (RAS → RAF → MEK → ERK) that transmits growth factor signals from the cell surface to the nucleus, regulating cell proliferation, differentiation, and survival. Constitutive activation (e.g. via KRAS mutations) drives tumourigenesis.
DPI (Dry Powder Inhaler): A device that delivers medication as a fine dry powder to the lungs upon inhalation. Used for inhaled treprostinil formulations (TYVASO DPI, YUTREPIA).
Soft Mist Inhaler: An inhaler that generates a slow-moving aerosol mist, potentially reducing airway irritation and cough compared to dry powder inhalers. UTHR's Tresmi uses this approach for treprostinil delivery.
Type A Meeting: An urgent FDA meeting to resolve critical issues that are blocking or have stopped drug development (e.g. clinical hold, study design dispute). Typically scheduled within 30 days of request. Distinguished from Type B meetings (60-day standard development discussions) and Type C meetings (90-day general advisory).
Functional Cure: A treatment outcome in which the disease is controlled to the point of clinical insignificance (e.g. normal life expectancy, absence of symptoms) without complete eradication of the underlying cause. Distinct from a true cure, as the disease mechanism may persist at a subclinical level.